



EVERGREEN COMMUNITY CARE INC. EMERGENCY PREPAREDNESS POLICY AND PLAN

Comprehensive Policy for All Emergencies in Canadian Residential Group Homes

Aligned to: Accreditation Canada (Qmentum) — CARF Standards — MCFD SHSS — BC OHS Regulation — BC Fire Code — Emergency Management BC — Health Canada

Document Reference	ECC-EP-2026-002
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Approved By	Executive Director
Effective Date	July 1, 2026
Review Date	July 1, 2027 — Annual review required; also triggered by any serious incident, new provincial directive, or accreditation finding
Applies To	All staff at all Evergreen Community Care residential group home programs for children and youth; all Program Coordinators; all Program Managers; Executive Director; Board of Directors
Accreditation Alignment	Accreditation Canada (Qmentum) — Required Organizational Practices (ROPs) including Emergency Preparedness; CARF International Standards — Section 1.J (Safety Management); MCFD Standards for Staffed Children's Residential Services (SHSS) — Standard F.4; BC OHS Regulation Part 4 (s.4.13–4.16); BC Fire Code Division B Part 2; Emergency Management BC (EMBC); Health Canada Communicable Disease Standards
Canadian Legal Framework	BC Emergency Program Act (SBC 1996, c.111); Workers Compensation Act (RSBC 1996, c.492); Community Care and Assisted Living Act (CCALA); Child, Family and Community Service Act (CFCSA); Emergency Management Act (Canada); Health Protection and Promotion Act principles; Controlled Drugs and Substances Act (CDSA)
Emergency Categories Covered	This plan covers all 7 emergency categories applicable to Canadian residential group homes for children and youth: (1) Fire; (2) Natural and



	Weather Emergencies; (3) Utility and Infrastructure Failures; (4) Facility Security Emergencies; (5) Medical and Clinical Emergencies; (6) Youth-Specific Emergencies; (7) Organizational and Staffing Emergencies
Related Documents	Behaviour Support Manual (ECC-BSP-2026-001); Medication Administration Manual (ECC-MED-2026-001); OHS Program Binder (ECC-OHS-POL-2026-002); RC Program Manual v4 (ECC-RC-POL-2026-003); PC Manual (ECC-PC-MAN-2026-001); PM Manual (ECC-PM-MAN-2026-001)

⚠ This is a CONTROLLED DOCUMENT. All staff must read and sign the acknowledgement (Appendix L) before their first shift. Emergency contacts and the evacuation plan are posted throughout the facility at all times. Every staff member can locate and explain the emergency procedures for their assigned area without reference to this document.



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PART 1 — Governance, Accreditation Alignment, and Roles

1. POLICY STATEMENT AND GOVERNING PHILOSOPHY

Evergreen Community Care Inc. is committed to the safety, security, and wellbeing of every child and youth in its care and every staff member working in its programs during any emergency. This Emergency Preparedness Policy and Plan establishes a comprehensive, accreditation-aligned framework for preventing, preparing for, responding to, and recovering from every type of emergency that can reasonably be anticipated in Canadian residential group homes for children and youth.

Emergency preparedness is not a compliance exercise. It is a core expression of our duty of care to the children and youth placed in our homes by MCFD. A residential care program that has not prepared for emergencies has not fulfilled its fundamental obligation to the young people in its care.

Governing Principle	Application in This Program
Safety before property	In every emergency, the safety of children and youth comes first, then staff safety. Property, records, and operations are secondary.
Prepare before you need to	Emergency procedures are practised regularly so they are automatic under stress. Training, not reading, creates competence in a crisis.
Every child has an individual emergency plan	Each youth's mobility, medical, communication, and behavioural needs are assessed and documented before an emergency occurs. Generic procedures are the minimum; individualized plans are the standard.
Culturally safe emergency response	Emergency response is delivered in a manner that is safe for Indigenous youth, LGBTQ2S+ youth, youth with disabilities, and youth from diverse cultural backgrounds. Dignity is maintained in every emergency.
Transparent escalation and communication	Every emergency activates a clear, practiced notification chain. No staff member manages a significant emergency alone. Escalation is required, not optional.
Learn from every event	Every drill, every near-miss, and every actual emergency is debriefed and documented. The lessons become practice improvements.
Accreditation and regulatory compliance	This program meets or exceeds all applicable accreditation standards (Accreditation Canada Qmentum, CARF) and regulatory requirements (BC OHS Reg., BC Fire Code, MCFD SHSS, RCR) for emergency preparedness.

2. CANADIAN AND BC LEGISLATIVE FRAMEWORK

Legislation / Standard	Jurisdiction	Key Emergency Preparedness Requirements
Emergency Program Act (SBC 1996, c.111)	BC Provincial	Establishes the provincial emergency management framework; all residential care programs must align with local government emergency plans; mandatory evacuation orders must be followed immediately
Workers Compensation Act (WCA)	BC Provincial	WCA s.115: employer duty to establish and maintain emergency preparedness program; WCA s.3.12: worker right to refuse unsafe work without retaliation
BC Occupational Health and Safety Regulation (OHS Reg.) Part 4	BC Provincial	OHS Reg. s.4.13: written emergency procedures required for all foreseeable emergencies; s.4.14: workers trained on emergency procedures; s.4.16: emergency drills at regular intervals
BC Fire Code (Division B, Part 2)	BC Provincial	Fire safety plan required for all Group B occupancies (residential care); quarterly fire drills minimum; annual unannounced drill; smoke detectors in all sleeping areas; CO detectors on all occupied floors; fire extinguisher annual inspection
Community Care and Assisted Living Act (CCALA)	BC Provincial	Licensing conditions include emergency preparedness; Health Authority Licensing Officer may inspect emergency records at any time; non-compliance may result in licence conditions, suspension, or cancellation
Residential Care Regulation (RCR)	BC Provincial	Safety and emergency practices must be



Standard F.4		documented; emergency equipment maintained; drills conducted; staff trained
MCFD Standards for Staffed Children's Residential Services (SHSS), April 2026	BC Provincial / MCFD	Standard F.4: written emergency procedures; drills at regular intervals; staff trained; youth emergency information maintained. Standard C.1: environmental safety. Standard G.3: staff emergency training required before unsupervised duty
Child, Family and Community Service Act (CFCSA)	BC Provincial	s.70: children in care have the right to physical safety at all times; s.14: mandatory reporting of any harm to a child in care, including harm during or arising from an emergency
Emergency Management Act (Canada)	Federal	National emergency management framework; programs must cooperate with federal emergency management authorities during declared national emergencies
Health Canada — Communicable Disease Standards	Federal	Standards for infection prevention and control in residential settings; pandemic preparedness guidelines; reporting obligations for designated communicable diseases
Controlled Drugs and Substances Act (CDSA)	Federal	Controlled substances must be secured and accounted for even during emergencies; losses must be reported to police and Health Canada

3. ACCREDITATION STANDARDS ALIGNMENT

Evergreen Community Care maintains compliance with the following accreditation standards for emergency preparedness. This section maps key accreditation requirements to the relevant sections of this plan.

3.1 Accreditation Canada (Qmentum) — Required Organizational Practices (ROPs)

Qmentum ROP / Standard	Requirement	Section of This Plan
ROP: Workplace Violence Prevention	Organizations must have a written workplace violence prevention program; staff trained; incidents reported and investigated	Sections 33, 53
ROP: Infection Prevention and Control	Organizations must have a written infection prevention and control program; outbreak response procedures; staff competency in standard precautions	Sections 38, 39
ROP: Safe Use of Blood and Blood Products (where applicable)	Procedures for safe handling of blood; exposure response	Section 35
Service Excellence Standard 1.3 — Physical Environment	Safe, accessible physical environment; emergency exits; hazard control; inspections documented	Sections 7, 8, 9, Appendix I
Service Excellence Standard 1.4 — Emergency Preparedness	Written emergency plan; staff trained; drills conducted at regular intervals; plan reviewed annually	Sections 55, 56, 57, 58
Service Excellence Standard 1.5 — Safety Culture	Organization fosters a safety culture; incidents and near-misses reported and reviewed; corrective action tracked	Sections 51, 52, 58
Leadership Standard 10 — Governance Responsibilities	Board ensures the organization has adequate resources and plans for emergency preparedness	Section 4
Medication Management Standards — Emergency Access to Medications	Medications accessible during emergencies; reconciliation after emergency relocation	Sections 9, 10, 12



3.2 CARF International Standards — Section 1.J (Safety Management)

CARF Standard	Requirement	Section of This Plan
1.J.1 — Environmental Safety	Written policies and procedures for environmental safety; regular inspections; hazard identification and correction	Sections 7, 8, Appendix I
1.J.2 — Emergency Preparedness	Written emergency procedures for all foreseeable emergencies; staff trained and evaluated; drills documented; annual review	All Parts 3–11
1.J.3 — Fire Safety	Fire safety procedures; regular drills; fire safety equipment maintained; evacuation plans posted	Part 3, Appendices C, G
1.J.4 — Infection Control	Infection prevention policies; outbreak response; communicable disease procedures	Sections 38, 39
1.J.5 — Security	Security procedures for facility and for the persons served; personnel trained in security procedures	Part 6
1.J.6 — Hazardous Materials	WHMIS/GHS compliance; SDS accessible; staff trained; spill response procedures	Section 29, Appendix D

3.3 Canadian Child Welfare Accreditation Framework (CCWAF)

CCWAF Domain	Requirement Relevant to Emergency Preparedness	Section of This Plan
Domain 3 — Safety of Children and Youth	Emergency policies ensure the physical safety of every child in care; individualized emergency plans for each child	Sections 10, 11
Domain 4 — Trauma-Informed Care	Emergency response is trauma-informed; children are not re-traumatized by emergency procedures; post-emergency support is therapeutic	Sections 51, 53
Domain 6 — Governance and Leadership	Board and leadership oversight of emergency preparedness; annual review submitted to the Board	Sections 4, 58
Domain 7 — Quality Improvement	Emergency preparedness is part of the quality improvement cycle; lessons from emergencies drive system improvements	Section 58



4. ROLES AND RESPONSIBILITIES

Role	Emergency Preparedness Responsibilities
Youth Care Worker (YCW) — First Responder	Implement the relevant emergency procedure immediately upon discovery; prioritize youth safety; account for all youth in their assigned area; call 911 if directed or if unable to reach the shift lead; administer first aid within certification; follow shift lead direction; document observations contemporaneously; never leave youth unattended during an emergency
Shift Lead — On-Site Incident Commander	Assume command of the facility-level response immediately upon discovery or notification; direct all staff; implement the correct emergency procedure from this plan; call 911 and the on-call supervisor; maintain an account of all youth and staff; transfer command to external authority (fire department, police, paramedics) upon their arrival while providing all relevant information; initiate all required notifications; document the event
Program Coordinator (PC)	Maintain all emergency preparedness documentation (Appendix I inspections, drill records, youth emergency profiles, 72-hour supply inventory); activate emergency staffing resources; support the shift lead from off-site; notify MCFD and external agencies as required; conduct monthly drills and training sessions; conduct post-emergency debrief; submit all required reports
Program Manager (PM)	Hold organizational accountability for emergency preparedness at the program level; approve and review this plan annually; respond to all significant emergencies; notify the ED; liaise with MCFD, the Licensing Officer, and accreditation bodies; submit all CI Reports and WorkSafeBC reports; lead the annual Emergency Preparedness Review; ensure all staff are trained and authorized; respond to all accreditation findings related to emergency preparedness
Executive Director (ED)	Approve this plan annually; receive notification of all significant emergencies; authorize any suspension of normal operations; liaise with the Board, MCFD, accreditation bodies, and media (ED-only authority); ensure adequate organizational resources for emergency preparedness; present the annual emergency preparedness summary to the Board
Board of Directors	Governance accountability for organizational emergency preparedness; receive the annual Emergency Preparedness Review; ensure the organization is resourced for emergency preparedness; review any significant emergency event at the next scheduled Board meeting
External Authorities — Fire, Police, Paramedics, Public Health	Upon arrival, external authorities assume command of their area of jurisdiction; all staff follow their direction; shift lead provides complete information (number of occupants; youth with special needs; location of hazard); program cooperates fully with all external authority investigations



5. EMERGENCY CLASSIFICATION SYSTEM

Evergreen Community Care uses a three-level emergency classification system for all emergency events. The classification determines the notification chain, response authority, and documentation requirements.

Level	Definition	Notification and Authority
Level 1 — Facility Emergency	An emergency that can be managed by on-site staff with existing resources and does not require service disruption or relocation. Examples: minor fire alarm (no fire found); short power outage; minor injury; a brief lockdown resolved quickly.	Shift lead manages; PC notified within 1 hour; documented in shift log; CI Report if youth or staff injury
Level 2 — Program Emergency	An emergency that requires off-site support, external resources, or temporary modification of service. Examples: extended power outage; significant medical emergency; missing youth; extended lockdown; communicable disease outbreak.	Shift lead initiates; PC notified immediately; PM notified within 30 minutes; MCFD notified as required; CI Report within 24 hours; ED notified same day
Level 3 — Organizational Emergency	An emergency that requires evacuation, relocation, suspension of services, or has potential liability, media, or accreditation implications. Examples: fire requiring evacuation; death in care; abduction; pandemic; building declared unsafe.	All levels notified simultaneously; ED activates organizational response; Board notified; MCFD notified; Licensing Officer notified; legal counsel engaged; CI Report within 24 hours; accreditation body notified if required by standards

6. EMERGENCY CONTACT DIRECTORY

This directory is posted at every telephone in the program, the main entrance, and the shift communication area. Reviewed quarterly. Updated within 24 hours of any change. The version date is displayed prominently.

EMERGENCY SERVICES	NUMBER
911 — fire, police, ambulance — all life-threatening emergencies	911



BC Poison Control Centre (24/7)	1-800-567-8911
Health Link BC — nurse advice line (24/7)	8-1-1
BC Emergency Management Division (24/7)	1-800-663-3456
WorkSafeBC — serious incident reporting (24/7)	1-888-621-7233
MCFD Provincial Centralized Screening (24/7)	1-800-663-9122
BC Crisis Line (24/7)	1-800-784-2433
Trans Lifeline (24/7)	1-877-330-6366
Kids Help Phone (24/7)	1-800-668-6868
Representative for Children and Youth (RCY)	1-800-476-3933
PROGRAM CONTACTS	
On-Call Supervisor — see posted weekly schedule	[Weekly schedule posted]
Program Coordinator — [Name]	[Cell number]
Program Manager — [Name]	[Cell number]
Executive Director — [Name]	[Cell number]
MEDICAL AND PHARMACY	
Program Pharmacy — [Name]	[Phone number]
On-Call Physician / NP — after hours	[Number]
Nearest Hospital Emergency — [Name and address]	[Phone number]
UTILITIES AND BUILDING	
BC Hydro — power outage reporting	1-888-769-3766



FortisBC — gas emergency (24/7)	1-800-663-9911
Local Police — non-emergency	[Number]
Building Owner / Property Manager — [Name]	[Number and after-hours]
HVAC / Maintenance Emergency	[Number]
FACILITY REFERENCE	LOCATION
Gas shutoff valve	[Describe location]
Electrical panel / main breaker	[Describe location]
Water main shutoff	[Describe location]
Evacuation assembly point	[Describe — minimum 15m from building]
First aid kit	[Describe location]
AED	[Describe location]
Fire extinguisher(s)	[Describe locations]
Emergency documentation package	[Describe location — accessible to shift lead]

PART 2 — Facility Preparedness and Prevention

7. FACILITY PREPAREDNESS REQUIREMENTS

7.1 Monthly Inspection — Priority Framework

The Program Coordinator completes the Monthly Safety and Emergency Inspection (Appendix I) by the last business day of each month. Every deficiency is assigned a priority level:

Priority	Definition	Resolution
Priority 1 — Immediate	Imminent threat to life; program cannot operate safely with this deficiency present	Resolved immediately; if unresolvable, PM notified for service suspension decision; youth moved to a safe area
Priority 2 — Urgent	Significant safety risk requiring rapid response	Resolved within 48 hours; PM notified same day
Priority 3 — Routine	Safety concern that should be addressed promptly	Resolved within 14 days; tracked on inspection report

7.2 Physical Facility Requirements — Emergency Standards

Requirement	Standard — All Programs
Fire detection — smoke alarms	Installed in every sleeping room, hallway, and common area; hardwired with battery backup preferred; interconnected so that activation of one activates all; tested monthly; batteries replaced annually (battery-powered models)
Carbon monoxide detectors	Installed on every occupied level; within 5 metres of each sleeping area; combined smoke/CO units acceptable; tested monthly
Fire extinguishers	Type ABC; minimum one per floor; visible and accessible; within 10 metres of any area; annual professional inspection; monthly visual check (pressure gauge, pin, seal)
Emergency exit lighting	Battery backup emergency lighting on all exit routes; activates automatically on power failure; tested monthly
Exit signage	All exit signs illuminated; clearly visible; tested monthly
Evacuation routes	Clear and unobstructed at all times; minimum two means of egress from every occupied floor; exterior paths clear of ice, snow, and obstruction



Automatic External Defibrillator (AED)	Mounted prominently; pads and battery in-date; all staff trained in use; checked monthly
First aid kit	WorkSafeBC-compliant Level 1 or 2 kit; accessible; fully stocked; checked monthly; restocked immediately after use
Hot water temperature	Maximum 49°C at all taps (RCR April 2026 amendment); thermostatic mixing valves installed where required; tested monthly with thermometer
Ligature risk assessment	Annual assessment; documented; modifications implemented per MCFD direction and Standard C.1
Security locks and entry management	All exterior doors lockable from inside; peepholes or intercom at main entrance; window locks functional
Chemical storage (WHMIS)	All hazardous products stored in designated, locked storage; SDS binder current and accessible; separate from food and medications

8. EMERGENCY EQUIPMENT STANDARDS

Equipment	Standard	Check Frequency	Responsible
Smoke detectors	All locations; test by button press	Monthly	PC via Appendix I
CO detectors	All occupied levels; test by button press	Monthly	PC via Appendix I
Fire extinguishers — visual	Green zone; pin and seal intact; no damage	Monthly	PC via Appendix I
Fire extinguishers — professional	Annual inspection; tag updated	Annual	PM arranges
Emergency exit lighting	Activation test — trip the breaker or test button	Monthly	PC via Appendix I
AED — pads expiry	Pads not expired; within-date indicator	Monthly	PC via Appendix I
AED — battery / indicator	Battery indicator shows ready; annual professional check	Monthly	PC via Appendix I
Emergency flashlights	All functional; batteries fresh	Monthly	PC via Appendix I
Battery radio	Receives AM/FM and emergency broadcasts	Monthly	PC via Appendix I
First aid kit	Fully stocked per kit	Monthly and	PC via Appendix I



	specification; nothing expired	after every use	
EpiPen (where applicable)	Not expired; accessible outside medication cabinet	Monthly	PC
Naloxone kit (where applicable)	Not expired; accessible outside medication cabinet	Monthly	PC
Emergency contact directory	All numbers confirmed current; posted	Quarterly	PC
72-hour supply inventory	Quantities adequate; nothing expired	Monthly	PC via Appendix E
Evacuation plan	Posted on every floor; current version	Annual update; immediate on any floor plan change	PC
Youth Emergency Profiles (Appendix F)	Current for all residents; updated within 30 days of any ISP change	Monthly	PC

9. 72-HOUR EMERGENCY SUPPLY INVENTORY

A 72-hour emergency supply inventory is maintained at every program at all times. Quantities are calculated for all current residents plus scheduled staff. The inventory is reviewed and restocked monthly using Appendix E. The PM approves the budget for emergency supplies annually.

Supply Category	Required Items	Quantity Basis
Water	Sealed bottled drinking water; water purification tablets as backup	Minimum 4 litres per person per day x 3 days
Food — non-perishable	Canned goods (with manual can opener); protein bars; crackers; ready-to-eat items; consideration for dietary restrictions and allergies of all residents	3 meals per person per day x 3 days
Medications	72-hour supply of all current medications for all residents; stored in a portable medication kit maintained by the PC; updated within 48 hours of any prescription change	Per each youth's current Individual Medication Profile
First aid	First aid kit (backup if primary is wall-mounted); extra gloves; extra bandages; CPR face shield	Per kit specification; extra supply for extended emergency



Warmth and shelter	Emergency thermal blankets (mylar); extra blankets; warm clothing per person; hand warmers (winter months)	Minimum 1 thermal blanket + 1 warm blanket per person
Lighting and communication	LED flashlights with extra batteries; battery-powered or hand-crank radio; charged portable battery packs and cables for staff phones	Minimum 1 flashlight per floor + 1 per person; 1 radio
Sanitation	Toilet paper; hand sanitizer; nitrile gloves; garbage bags (large); wipes	3-day supply per person
Documents	Copies of Youth Emergency Profiles (Appendix F); Emergency Contact Directory; insurance information; blank incident report forms	One complete set in a waterproof container
Youth comfort and wellbeing	Activity books; cards; small games; comfort items per youth (documented in emergency profile); sensory tools for youth who need them	Sufficient for 3 days for all current residents
Special needs items	Mobility aid supplies; communication device chargers; specialized dietary items; infant/toddler supplies if applicable	Per each youth's individual needs
Cash	Small bills; ATMs may be offline during emergencies	Amount approved by PM annually
Cold weather / winter additions	Extra warm clothing layers; boots; toques and mitts; road salt; snow removal tools	Per person; seasonal addition from October through April

10. YOUTH EMERGENCY PROFILES AND GO-BAGS

10.1 Youth Emergency Profile — Required Content

Every youth in residence has a current Youth Emergency Profile (Appendix F) in the Emergency Documentation Package. This profile is updated within 30 days of any ISP change and is reviewed monthly by the PC. It contains everything that emergency responders and receiving facilities need to know about this youth.

Profile Section	Required Content
Identity	Legal name; preferred name; pronouns; date of birth; photo (current)
Emergency contacts	MCFD social worker (name, office phone, cell); emergency contacts (authorized persons only: name, relationship, phone); next of kin (as directed by MCFD)
Medical alerts	All known allergies (including severity and reaction type); current

	medical diagnoses relevant to emergency response; EpiPen prescribed (yes/no); Naloxone prescribed (yes/no); any medical equipment in use
Current medications	All current medications; dose; time-critical doses (must not be missed); prescriber and after-hours contact; pharmacy name and number
Mobility and physical needs	Can this youth evacuate independently by stairs and on foot? Equipment required (wheelchair, walker, crutches); staff required for evacuation; weight limit considerations for carry evacuation
Communication needs	Primary language; AAC device; communication board; cognitive level for understanding instructions during stress; significant communication disabilities that affect emergency response
Behavioural response to emergencies	Known triggers in emergency situations (loud alarms, crowds, uniforms, darkness); likely behavioural responses (elopement risk, physical aggression, self-harm risk, freezing, dissociation); effective de-escalation approaches during emergency stress; approaches to avoid
Cultural and identity considerations	Indigenous Nation affiliation; cultural or spiritual practices relevant to emergency response; gender identity and privacy considerations during emergency sheltering; religious observance that may affect emergency decisions
Go-bag contents for this youth	Items that must travel with this youth in any evacuation or relocation

10.2 Go-Bag Contents — Per Youth

Go-Bag Item	Standard
Medications — 72-hour supply	Pre-packed in a labelled, secure container; updated within 48 hours of any prescription change; controlled substances require secure transport per CDSA
Copy of Youth Emergency Profile	Waterproof container; current version
Health card and personal ID	BC Services Card; any other government ID on file
Comfort items	Youth-identified items documented in the emergency profile; practical and portable
Changes of clothing	Minimum 2 changes; weather-appropriate; seasonally reviewed
Phone charger / portable battery	For any youth who has a phone
Communication device or board	For any youth who uses AAC



Sensory or calming items	As documented in the BSP; fidget, headphones, specific comfort object
Special medical supplies	Any medical supplies specific to this youth's needs (ostomy supplies, feeding supplies, etc.)

11. EVACUATION PLANNING — ALL SCENARIOS

11.1 Evacuation Plan Requirements

Requirement	Standard
Written evacuation plan	Prepared for every floor of every program facility; reviewed annually; updated within 30 days of any facility change
Contents	Floor plan of every floor; primary and secondary evacuation routes; assembly point; designated staff roles during evacuation; youth-specific notes per Emergency Profile
Posting	Posted on every floor; at every exit; in the shift communication area; in the Emergency Documentation Package
Assembly point	Minimum 15 metres from the facility; accessible for wheelchairs and mobility aids; away from fire apparatus routes; marked with a permanent sign; alternative assembly point identified if primary is blocked
Alternate exit for non-ambulatory youth	Evacuation chair maintained where any youth cannot independently use stairs; trained staff assigned; 'defend in place' protocol documented with fire department for any youth for whom evacuation is not possible
Night scenario	Specific night evacuation procedure accounts for sleeping youth; staff know which doors to knock on first; timing of night drill demonstrates capability

11.2 Evacuation by Scenario

Scenario	Evacuation Decision
Fire — alarm activation	Evacuate immediately — every alarm is treated as real until confirmed otherwise by the fire department. Do not investigate first. Evacuate first.
Earthquake — after shaking stops	Evacuate if: structural damage visible; gas smell; fire; or fire department directs. Shelter in place if: no damage; no hazards; fire department has not directed evacuation.



Gas leak	Evacuate immediately; do not stop to collect belongings; do not use electrical switches; evacuate to upwind assembly point
Flood — Evacuation Order issued by authorities	Evacuate immediately per the order; follow designated evacuation routes to alternate facility
Wildfire — Evacuation Order issued	Evacuate immediately; take go-bags; notify MCFD; proceed to alternate facility
Structural damage	Evacuate if any structural compromise is visible or suspected; do not wait for confirmation; re-enter only when authorized by fire department or structural engineer
Intruder / lockdown	Shelter in place (lockdown) unless directed to evacuate by police; do not evacuate during a lockdown without police direction
CBRN (chemical, biological, radiological, nuclear)	Shelter in place or evacuate per direction of emergency services; do not self-evacuate into a contaminated area

12. BUSINESS CONTINUITY AND ALTERNATE FACILITY

12.1 Pre-Identified Alternate Facility

Each program maintains a written agreement with a pre-identified alternate facility where youth can be temporarily relocated during an extended emergency. This is confirmed annually. The agreement is in the Emergency Documentation Package.

Alternate Facility Element	Requirement
Facility identification	Name, address, and contact; confirmed suitable for residential care of children and youth; accessible; adequate capacity
Written agreement	Letter of understanding or formal agreement; confirmed annually by PM; copy in Emergency Documentation Package
Transportation plan	Vehicles, drivers, and route to the alternate facility identified; vehicle capacity sufficient for all youth; mobility aid transport confirmed
MCFD approval	MCFD is notified immediately upon activation; PM obtains MCFD direction on any placement change required
Medications at alternate facility	Go-bags with 72-hour medication supply travel with each youth; PC coordinates with pharmacy for extended supply if needed
Communication with families	MCFD social workers notify families per MCFD protocol; program does not contact families independently without MCFD direction except in life-threatening emergency



12.2 Business Continuity Activation

Step	Action	Authority
1. PM declares Level 3 emergency	PM assesses that the primary facility cannot be safely used; Business Continuity activated	PM — ED informed simultaneously
2. Notify MCFD immediately	PM notifies all MCFD social workers for current youth and the MCFD Contract Manager; provides alternate location, estimated duration, and any changes to youth placement	PM
3. Notify Licensing Officer	PM notifies the Health Authority Licensing Officer of the service disruption	PM
4. Transport youth to alternate facility	All youth transported with their go-bags; Emergency Documentation Package travels with the shift lead; medications secured per CDSA	Shift lead + all available staff
5. Notify accreditation body if required	If the emergency affects accreditation standing, PM notifies the accreditation body per their standards	PM — ED approves
6. Restore or relocate permanently	PM and ED assess return timeline; MCFD and Licensing Officer kept informed of all developments	PM + ED



PART 3 — Category 1: Fire Emergencies

13. FIRE PREVENTION POLICY

Fire Prevention Area	Policy and Standard
Smoking	Prohibited inside all facilities and within 6 metres of any entrance, window, or air intake; smoking materials disposed of in designated exterior receptacles only
Kitchen and cooking	Cooking areas never left unattended with heat on; stovetop cleaned regularly to prevent grease accumulation; kitchen exhaust fan used whenever cooking; no personal cooking appliances in bedrooms without PM approval
Electrical safety	Extension cords are temporary only; no overloading of outlets; no daisy-chaining of power bars; electrical panels accessible and clear; any sparking, flickering lights, or burning smell reported to PC immediately
Open flames and candles	Prohibited without PM approval; cultural or ceremonial use with supervision requires risk assessment and PM authorization; never left unattended
Storage	All exit routes and corridors clear at all times; combustible materials stored away from heat sources; no storage within 1 metre of electrical panels or heat sources
Dryers	Lint trap cleaned before every load; dryer duct professionally inspected annually; dryer never left running when building is unoccupied
Holiday decorations	CSA-certified lights only; lights not left on when building is unoccupied; artificial trees preferred; no real trees without PM approval; nothing placed near heat sources
Regular fire safety inspection	Monthly facility inspection per Appendix I includes all fire safety items; any deficiency resolved within the priority timeline

14. FIRE EMERGENCY RESPONSE — RACE AND PASS

⚠ Every staff member memorizes RACE before their first unsupervised shift. It is posted in every room. A fire alarm is always treated as real until the fire department confirms otherwise. Do not investigate first. Evacuate first.

RACE — memorize and	Action
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practise	
R — RESCUE	Remove anyone in immediate danger. One attempt only. Do not re-enter a burning room. Protect yourself.
A — ALARM	Activate the nearest fire alarm pull station. Call 911 immediately — give your address, number of occupants, and any known information about the fire or anyone trapped.
C — CONTAIN	Close all doors between the fire and the rest of the building. Closed doors slow fire and smoke spread significantly. Do not lock doors.
E — EVACUATE	Evacuate all youth and staff to the assembly point using the posted evacuation plan. Take the Emergency Documentation Package. Account for all persons.

A trained staff member may attempt to extinguish a small fire (wastebasket-sized or smaller) using PASS after RACE has been initiated, only if the staff member has a clear exit behind them and has received practical fire extinguisher training:

PASS — fire extinguisher use	Action
P — PULL	Pull the safety pin from the handle
A — AIM	Aim the nozzle at the BASE of the fire, not the flames
S — SQUEEZE	Squeeze the handle to discharge
S — SWEEP	Sweep from side to side at the base of the fire until extinguished or extinguisher is empty

⚠ If the fire is not extinguished within 10 seconds of PASS or is producing significant smoke: ABANDON the attempt and evacuate immediately. Never risk your life for property. Never re-enter a burning building.

15. EVACUATION PROCEDURE — STEP BY STEP

Step	Action	Responsible
1. Activate alarm and call 911	Pull the nearest alarm pull station; call 911 immediately; give address, number of occupants, any information about fire or trapped persons	First staff to discover fire or alarm
2. Assign staff to	Shift lead immediately assigns each staff	Shift lead



youth	member to specific youth; assigns one staff to sweep each floor; assigns one staff to retrieve the Emergency Documentation Package	
3. Begin evacuation	Move all youth to the nearest unobstructed exit; close all doors as you go — do not lock them; do not use elevators; assist youth with mobility needs per their Emergency Profile	All staff — each assigned to specific youth
4. Sweep each floor	Designated sweep staff knock on every door and shout 'FIRE — EVACUATE'; check all rooms, bathrooms, and closets; close doors after checking; report all-clear for each floor to the shift lead	Designated sweep staff — one per floor
5. Proceed to assembly point	All youth and staff proceed to the designated assembly point; no one re-enters for any reason	All staff and youth
6. Account for all persons	Shift lead takes full account of every youth and staff member at the assembly point; any missing person reported immediately to the fire department on arrival	Shift lead — using Emergency Documentation Package
7. Meet the fire department	Shift lead meets fire department at the street or driveway; provides: address; number of occupants; any missing persons; location of fire if known; any hazards (oxygen, compressed gas, medications)	Shift lead
8. Do not re-enter	No staff or youth re-enters the building for any reason until the senior fire officer declares it safe; do not retrieve belongings	All staff — shift lead enforces
9. Notify PC and PM	Shift lead calls PC immediately after all persons are accounted for	Shift lead
10. Document	Shift lead begins incident documentation immediately; records time of alarm, time all persons at assembly point, any injuries, fire department response time, all-clear time	Shift lead; PC follows up with formal CI Report

16. FIRE DRILLS — REQUIREMENTS AND DOCUMENTATION

Fire Drill Requirement	Standard — Accreditation and Regulatory Aligned
Quarterly announced drills	Minimum 4 fire drills per year; one per quarter; all staff and youth present participate; full evacuation to assembly point; timed from alarm activation to all persons accounted for at assembly point; documented on Appendix G immediately after the drill



Annual unannounced drill	Minimum 1 unannounced fire drill per year; PM or PC observes and evaluates; conducted at a non-peak time (evening, night, or weekend) to simulate realistic conditions; documented on Appendix G
New staff orientation drill	Every new staff member completes the fire evacuation walkthrough (exits, assembly point, their role) during their first shift; participates in the next scheduled drill
Night drill	At minimum 1 drill per year simulates night conditions (may be the unannounced drill); evaluates staff ability to wake and evacuate youth; evaluates time to evacuate from sleeping
Documentation requirement	Every drill documented on Appendix G within 24 hours: date; time; announced or unannounced; number of staff and youth; evacuation time; issues identified; corrective actions assigned
Post-drill debrief	Brief debrief with all participating staff immediately after every drill: what went well; what needs improvement; youth-specific issues; corrective actions with owner and date
Corrective action follow-through	Every deficiency identified in a fire drill generates a corrective action with a named owner and target date; confirmed complete before the next drill or sooner; PM reviews all corrective actions monthly
Accreditation evidence	Fire drill records are maintained as accreditation evidence; available for Accreditation Canada, CARF, and Licensing Officer review at any time; retained minimum 3 years

PART 4 — Category 2: Natural and Weather Emergencies

17. EARTHQUAKE RESPONSE

British Columbia is in one of Canada's most seismically active regions. Earthquakes occur without warning. Every staff member must be able to execute the response without thinking.

17.1 During the Earthquake — Drop, Cover, Hold On

Action	Detail
DROP	Drop to the floor immediately. Do not try to run. Most injuries occur when people move during shaking.
COVER	Get under a sturdy desk or table. If no table: cover your head and neck with your arms; move to an interior wall away from windows and exterior walls.
HOLD ON	Hold on to your shelter. Do not let go. Do not stand until the shaking has completely stopped.
For youth in bed	If the youth is asleep: stay in bed and use the pillow to protect the head and neck. Do not move unless there is a hazard nearby.
Do not run outside	Most injuries during earthquakes occur when people run outside. Stay inside until the shaking stops.
Away from windows	Windows and glass are the greatest hazard during shaking. Move away from windows and exterior walls before the shaking starts if you have any warning.

17.2 After the Earthquake

Step	Action	Responsible
1. Account for all youth and staff	When shaking completely stops: call out to all youth and staff; physically check each person; provide first aid for any injuries; call 911 for serious injuries	Shift lead
2. Check for immediate hazards	Gas smell — evacuate immediately (see Section 25); structural damage — evacuate if visible cracks, leaning, or buckling; fire — initiate fire procedure; downed lines outside — keep everyone inside	Shift lead + all staff



3. Evacuate if unsafe	Any structural damage, gas smell, or fire: evacuate using evacuation plan; do not re-enter until fire department or structural engineer clears the building	Shift lead
4. Expect aftershocks	Aftershocks can occur for days or weeks; staff maintain readiness to Drop, Cover, Hold On again; communicate this to youth calmly	All staff
5. 72-hour supply activation	If utilities are disrupted: access the 72-hour emergency supply inventory; ration water and food; assess need to activate business continuity plan	Shift lead; PC advises
6. Notification chain	Shift lead notifies PC; PC notifies PM; PM notifies ED; CI Report as required by event level	Per notification chain

17.3 Earthquake Preparedness — Facility Requirements

- All heavy furniture (bookshelves, cabinets, TV stands) secured to walls with appropriate anti-tip brackets or straps
- Water heater strapped to the wall per BC Building Code
- Heavy objects stored on lower shelves; breakable items secured or stored low
- Gas shutoff valve location known to all shift leads and posted in the Emergency Contact Directory
- Wrench for gas shutoff mounted near the shutoff valve if applicable
- 72-hour emergency supplies maintained at all times

18. FLOOD AND SEVERE WEATHER RESPONSE

18.1 Flood Response — Four Alert Levels

Alert Level	Meaning	Required Action
Flood Watch	Flooding conditions are possible in the area	Monitor BC Wildfire and Emergency Services (emergencyinfobc.ca); review evacuation route; confirm 72-hour supplies; notify PC
Flood Advisory	Flooding is possible; conditions are developing	PC notifies PM; PM advises on preparedness steps; confirm alternate facility is available
Flood Warning	Flooding is expected; be ready to evacuate on	Prepare go-bags; move



	short notice	essential records and medications to upper floors; activate business continuity plan standby; notify MCFD
Evacuation Order — mandatory	Leave immediately; life threat	Evacuate immediately to alternate facility; take Emergency Documentation Package and go-bags; call MCFD and PM simultaneously

18.2 Severe Weather — Windstorm, Thunderstorm, Extreme Cold

- Sign up for Alertable or the provincial emergency alert system for the program's region (available through the BC government website)
- Before severe weather: confirm 72-hour supplies are adequate; confirm vehicles are fuelled; confirm staffing plan if roads become impassable; bring outdoor furniture and equipment inside
- During windstorm: youth remain inside; windows and doors closed; stay away from windows; outdoor activities cancelled
- During thunderstorm: no outdoor activities; stay away from metal objects, windows, plumbing; if a youth is outdoors, move them inside immediately
- If roads are impassable: shift lead cannot leave until relief arrives; activate Emergency Staffing protocol (Section 46)

19. EXTREME HEAT AND HEAT EMERGENCY

The 2021 BC heat dome event demonstrated that extreme heat is a life-threatening emergency. Youth on certain medications (antipsychotics, stimulants, anticholinergics) are at significantly elevated risk.

Heat Emergency Action	Standard
Heat alert monitoring	Monitor Environment and Climate Change Canada alerts at weather.gc.ca ; sign up for emergency alerts; monitor the BC HealthLink heat alert system
Medication review during heat emergency	At the onset of any heat advisory: PC reviews all current medications for heat sensitivity (antipsychotics, anticholinergics, diuretics, stimulants); contacts pharmacy or prescriber for specific monitoring instructions during extreme heat
Active cooling measures	Air conditioning maximized; fans in all occupied rooms; cool water freely available; light breathable clothing; cool shower or bath available; outdoor activities cancelled between 10 AM and 4 PM



Interior temperature monitoring	Document indoor temperature every 2 hours during a heat event; if interior temperature exceeds 30°C: notify PM immediately; identify alternative cooled space
Alternative cooling location	PM pre-identifies an accessible air-conditioned alternative (community centre, library) before heat season; authorization required before transporting youth
Signs of heat exhaustion	Heavy sweating; cold, pale, clammy skin; rapid weak pulse; nausea; dizziness: move to cooler area; cool fluids; loosen clothing; call HealthLink (8-1-1) for guidance
Signs of heat stroke — EMERGENCY	Hot, red, dry or damp skin; rapid strong pulse; confusion; loss of consciousness: CALL 911 IMMEDIATELY; cool the person rapidly with wet cloths, cool water immersion, or ice packs on neck, armpits, groin; do not give fluids to an unconscious person

20. WINTER STORM AND EXTREME COLD

Winter Emergency	Response
Snowstorm — minor	Monitor roads; cancel non-essential outings; confirm next shift staff can travel safely; activate Emergency Staffing protocol if staff cannot travel (Section 46)
Snowstorm — major (roads impassable)	Shift staff shelters in place; activate Emergency Staffing protocol immediately; 72-hour supplies accessed; PM coordinates; MCFD notified if youth care is compromised
Extreme cold weather alert (wind chill -25C or below)	Youth do not go outdoors without full winter clothing and a time limit; outdoor activities cancelled; youth with poor cold sensitivity (neuropathy, circulation issues) require enhanced monitoring; check that all outdoor areas are accessible and safe
Ice storm	All exterior surfaces highly dangerous; no outdoor movement without slip-resistant footwear; emergency exits must remain accessible; facilities manager notified to treat entry areas with salt or sand
Heating system failure	See Section 28; 72-hour supplies include extra blankets and warm clothing; PM notified immediately; repair arranged as Priority 1
Frozen pipes	Do not attempt to thaw pipes with open flame; property manager notified immediately; water supply backup accessed from 72-hour supplies if needed

21. WILDFIRE AND EVACUATION ORDERS

Wildfires in BC have increasingly affected urban and suburban areas. This procedure applies to any program located in or near an area subject to wildfire risk.

Alert Level	Meaning	Action
Wildfire Watch (potential risk in area)	Conditions exist that could lead to wildfire; no immediate threat	Monitor BC Wildfire Service (bcwildfire.ca); review evacuation plan; confirm 72-hour supplies; notify PC of watch status
Evacuation Alert (be ready to leave in minutes)	Wildfire is approaching; evacuation may be required quickly	Pack go-bags immediately; prepare Emergency Documentation Package; confirm alternate facility destination with PM; notify all MCFD social workers of alert status; pre-position vehicles
Evacuation Order (leave immediately)	Immediate life threat	Evacuate immediately to pre-identified alternate facility; take all go-bags; call MCFD social workers and PM simultaneously; follow BC Emergency Management direction for evacuation route; register with local emergency management authority at destination

22. TORNADO AND SEVERE THUNDERSTORM

While tornadoes are less frequent in BC than on the prairies, they do occur in BC and are more common in Alberta and across the rest of Canada. Programs in provinces across Canada should follow this procedure.

Threat	Response
Tornado Watch (conditions favourable for tornado formation)	Monitor Environment Canada at weather.gc.ca; prepare to implement Tornado Warning procedure; ensure everyone knows the designated shelter area
Tornado Warning (tornado)	Immediately move all youth and staff to the lowest floor interior room or

has been sighted or indicated by radar)	hallway; away from all windows; get under a table or stairwell if possible; crouch low and cover head and neck; do not go outside; do not attempt to outrun a tornado in a vehicle
During a tornado	Stay in the designated shelter area; stay away from windows; do not go outside until the warning is lifted by Environment Canada
After a tornado	Check for injuries; check for structural damage, gas leaks, or fire; call 911 for any injuries; notify PC and PM; document all damage
Severe thunderstorm with lightning	No outdoor activities; stay away from metal objects, plumbing, and windows; unplug sensitive electronics; stay inside until the storm passes

23. POOR AIR QUALITY AND SMOKE EVENTS

Air Quality Health Index (AQHI)	Action
AQHI 1–3 (Low Risk)	Normal activities; no action required
AQHI 4–6 (Moderate Risk)	Youth with respiratory conditions: reduce prolonged outdoor exertion; ensure rescue inhalers are accessible; monitor for symptoms
AQHI 7–10 (High Risk)	All youth remain indoors; windows and doors closed; air conditioning on recirculate (not fresh air intake); all outdoor activities cancelled; enhanced monitoring for youth with asthma or respiratory conditions
AQHI 10+ (Very High Risk) or Special Air Quality Statement issued	Keep all youth inside with windows and doors sealed; towels at door gaps if smoke is heavy; HEPA air purifiers operating; contact prescriber for any youth with respiratory symptoms; document all precautions

- Monitor AQHI at aqhi.weather.gc.ca or through the province's public health system
- For wildfire smoke events: follow BC Centre for Disease Control (BCCDC) guidelines; N95 masks for any necessary outdoor movement; youth with asthma keep rescue inhalers immediately accessible
- For industrial air quality incidents: follow direction of local emergency management; shelter-in-place may be directed; seal windows and doors and turn off HVAC if directed

PART 5 — Category 3: Utility and Infrastructure Failures

24. POWER FAILURE

Duration	Response	Decision Authority
Brief (under 2 hours)	Emergency lighting activates; flashlights deployed; report outage to BC Hydro (1-888-769-3766); continue operations with adaptations; document in shift log	Shift lead manages
Extended (2–8 hours)	Activate battery radio for emergency broadcasts; check refrigerated medications (do not open unnecessarily); monitor facility temperature; notify PC; arrange for alternative lighting and heating if needed	PC notified; PM advised
Prolonged (over 8 hours)	PM decides whether to activate Business Continuity Plan; consider transfer to alternate facility; notify MCFD; contact property manager for generator or alternative power; refrigerated medications: contact pharmacy if above safe temperature	PM decides; ED informed; MCFD notified

24.1 Medications During Power Failure

- A closed refrigerator maintains safe temperature (2–8°C) for approximately 4 hours; a cooler with ice can extend this
- After 4 hours without power: contact the pharmacy before administering any refrigerated medication; do not assume it is still safe
- Electronic medical equipment (feeding pumps, CPAP, nebulizers): confirm battery backup; contact prescriber if equipment cannot operate
- Medication cabinet security: confirm the cabinet is still secure; manual monitoring if electronic alarms are offline

25. NATURAL GAS LEAK

Step	Action — in sequence
1. Detect	Signs of gas leak: smell of rotten eggs or sulphur (mercaptan); hissing near appliances or lines; dead vegetation near gas lines; dizziness or nausea inside
2. Do NOT operate any	Any spark can ignite gas; do not turn lights on or off; do not use



electrical switches	thermostats; do not use any appliance
3. Leave immediately	Get everyone out of the building; leave doors open as you exit; do not stop for belongings
4. Do NOT use phones inside the building	Move outside and at least 100 metres away before calling; cell phones can spark
5. Call FortisBC Gas Emergency from outside	1-800-663-9911 — from outside and away from the building; call 911 if anyone is injured or the leak is large
6. Do not re-enter	Do not re-enter until FortisBC and the fire department declare the building safe
7. Notify PC and PM from outside	PC notifies PM; PM activates Business Continuity Plan if re-entry is not possible same day

26. WATER DISRUPTION AND CONTAMINATION

Situation	Response
No running water — utility disruption	Activate 72-hour water supply (minimum 4 litres per person per day); notify property manager; notify PC; if disruption exceeds 12 hours: PM activated; consider alternate facility if water cannot be safely supplied within 24 hours
Boil Water Advisory issued by health authority	All water for drinking, brushing teeth, cooking, or ice must be boiled 1 minute (or bottled water used); notify all youth and staff immediately; post advisory notice; document all precautions; monitor public health authority for advisory end
Do Not Use Advisory (chemical or biological contamination)	Do not use water for any purpose; notify PC and PM immediately; activate 72-hour water supply; contact public health authority (8-1-1 or local health authority); follow all public health direction; document
Major plumbing failure — sewage backup	Isolate affected area immediately; contact property manager; if sewage contacts youth living areas: PM determines whether youth can remain; MCFD notified; consider temporary relocation
Flood water in the facility	Do not use any water from the tap if flooding has occurred — water supply may be contaminated; activate 72-hour water supply; evacuate if structural integrity is at risk

27. STRUCTURAL DAMAGE AND UNSAFE BUILDING



Signs Requiring Immediate Evacuation	Response
Visible cracks in load-bearing walls or foundation	Evacuate immediately; do not re-enter
Roof sagging, damage, or collapse risk	Evacuate immediately; do not re-enter
Floor or wall leaning, buckling, or shifting	Evacuate immediately; do not re-enter
Any structural member that has visibly moved, shifted, or cracked	Evacuate immediately; do not re-enter
Water damage to structural elements (prolonged flooding or roof leak)	PM determines; structural engineer assessment required before youth can remain

- Following evacuation: do not re-enter until the fire department or a qualified structural engineer declares the building safe
- PM notified immediately; PM contacts property owner and arranges professional assessment
- If the building is declared unsafe for occupancy: Business Continuity Plan activated immediately
- MCFD social workers notified within 2 hours of any emergency relocation

28. HEATING SYSTEM FAILURE

Temperature	Response
Below 18°C interior	Notify property manager immediately; deploy extra blankets from 72-hour supplies; notify PC
Below 15°C interior	Notify PC and PM immediately; electric space heaters deployed if available (must not be left unattended and not placed near combustibles); evaluate whether youth can safely remain
Below 10°C interior or exterior temperature is extreme	PM determines immediate need to relocate youth to alternate facility; activate Business Continuity Plan; MCFD notified
Pipes at risk of freezing	Property manager notified immediately; outdoor taps wrapped; cabinet doors under sinks opened to allow warm air circulation; water allowed to drip slowly in extreme cold



29. SEWAGE BACKUP AND ENVIRONMENTAL CONTAMINATION

- Sewage in any living, sleeping, or food preparation area is a health emergency: isolate the area immediately; youth do not enter the contaminated area; property manager notified as Priority 1
- If sewage affects multiple areas of the facility: PM determines whether youth can remain in any part of the facility safely; MCFD notified
- Clean-up is conducted by qualified remediation professionals; staff do not clean sewage contamination without appropriate PPE
- Hazardous material spill (cleaning products, unknown chemicals): evacuate the affected area; identify the substance using the WHMIS SDS binder; call 911 if the substance is toxic or released in quantity; call Poison Control (1-800-567-8911) for guidance; do not re-enter until safe
- External environmental incident (transportation accident, industrial release, chemical cloud): follow direction of emergency services; shelter-in-place or evacuation as directed; close windows and doors and turn off HVAC for shelter-in-place

PART 6 — Category 4: Facility Security Emergencies

30. INTRUDER AND UNAUTHORIZED ENTRY

Situation	Response
Unknown person at the door — not a scheduled visitor	Do not open the door; identify through window or intercom; ask for ID and purpose of visit; if not expected and authorized by PC: do not admit; ask the person to contact PC or PM by phone
Person demanding entry — angry or threatening	Do not open the door under any circumstances; call 911 immediately if threats are made or forced entry is attempted; call PC; move all youth away from doors and windows; document all details (description, time, what was said)
Unauthorized person inside the facility	Do not confront the person; call 911 immediately; get all youth into a room and lock the door if possible; wait for police; do not put yourself between the intruder and the exit
Former staff or person with known access	Do not assume former access means current authorization; all staff whose employment is terminated have their access revoked immediately; treat unauthorized entry as above
Media or camera crew at the facility	Do not speak to any media; do not allow any filming of the facility or any youth; call PC immediately; ED is the only authorized spokesperson; direct all media to contact the ED

31. LOCKDOWN PROCEDURE

A lockdown is activated when there is an immediate threat to the safety of youth and staff from an external or internal threat that cannot be resolved by normal means. Lockdown means: everyone inside; all exterior access secured; police called.

Lockdown Step	Action
1. Announce lockdown	Shift lead announces lockdown to all staff immediately using established code word or clear announcement; do not announce on external radio channels
2. Lock all exterior doors and windows	Every exterior entrance secured; windows in all youth-occupied and common areas closed and locked; blinds or curtains closed
3. Move all youth to interior rooms	Away from all exterior windows and doors; interior hallways or rooms with locking doors preferred; get youth low and away from exterior walls



4. Call 911	Call 911 immediately if not already done; advise: number of persons in the building; nature of the threat if known; whether anyone is injured; the full address; stay on the line if safe to do so
5. Account for all persons	Account for every youth and staff member; if anyone is unaccounted for, advise police immediately when they arrive
6. Silence mobile phones	All phones on silent mode; communicate by text if possible; keep lines clear for 911
7. Do not open doors for anyone during lockdown	Not for parents; not for visitors; not for anyone claiming to be police until they provide badge number and you can visually confirm through a window or peephole; real police will identify themselves and provide verification
8. Wait for ALL CLEAR from police	All clear must come from police in person at the door or confirmed by PC/PM upon police confirmation; do not unlock until ALL CLEAR is received
9. Notify PC and PM	Shift lead notifies PC immediately after calling 911

32. BOMB THREAT

Bomb Threat — Step	Action
If threat received by phone	Stay calm; keep the caller talking as long as possible; note exact words; note any background sounds; note the caller's voice characteristics (gender, accent, tone); do not hang up; signal another staff member to call 911 from another phone
If threat received by written note or email	Do not handle the note unnecessarily; take a photo; call 911; notify PC
Do not search for a device yourself	Only trained police or explosive ordnance disposal personnel search for explosive devices
Evacuation decision	Police make the decision to evacuate or shelter in place; staff follow police direction; if police are not yet on-site and the threat is credible and specific: shift lead may initiate precautionary evacuation using the fire evacuation procedure
After evacuation	Do not allow re-entry until police clear the building; cooperate fully with police investigation; notify PC and PM

33. WORKPLACE VIOLENCE AND ASSAULT

Workplace violence in a residential care setting includes physical assault, verbal threats, intimidation, and any act that causes a worker to fear for their safety. This is both an OHS issue (WCA and OHS Reg. s.4.27) and a safety issue for youth.

Situation	Response
Youth physically assaults staff — during de-escalation failure	Follow de-escalation and restraint procedures per Behaviour Support Manual; call for backup; call 911 if the assault cannot be managed safely by staff; staff medical assessment after any physical assault; WorkSafeBC Form 7 if medical treatment required; CI Report
External person assaults staff or threatens staff	Call 911 immediately; do not attempt to physically intervene against an external attacker; secure youth inside; staff medical assessment; WorkSafeBC Form 7 if medical treatment required; CI Report
Verbal threats or intimidation by any person	Document all details; notify PC same shift; if threats are credible and ongoing: notify police; do not dismiss verbal threats as harmless; they are a precursor to physical violence
Domestic violence situation — visitor or family member poses threat	Call 911 immediately if the person is threatening or has a weapon; implement lockdown if needed; do not allow the threatening person inside; keep the youth safe inside; notify PC and MCFD
Staff-on-youth violence (any use of force outside the BSP framework)	Treat as a prohibited practice per the Behaviour Support Manual; separate the staff member from youth immediately; call PC and PM; MCFD notified same day; suspension pending investigation; mandatory reporting assessment under CFCSA s.14

34. VEHICLE INCIDENT AT OR NEAR THE FACILITY

Situation	Response
Vehicle impacts the building	Call 911 immediately; evacuate the affected area; check for injuries; assess structural damage per Section 27; do not approach the vehicle if gas or fluid is leaking and fire risk exists; notify PC and PM
Vehicle fire in the parking area or adjacent to the building	Call 911 immediately; if fire is near the building: activate fire evacuation procedure (Section 15); keep youth inside or at the assembly point away from the fire
Traffic accident near the facility involving a program vehicle	Call 911; ensure youth and staff safety first; do not move any vehicle or persons unless there is immediate danger; notify PM; WorkSafeBC report if any staff injury; CI Report if any youth injury
Hazardous material spill from a vehicle on or near the property	Do not approach; call 911; shelter-in-place or evacuate per emergency services direction; notify PC

**PART 7 — Category 5: Medical and Clinical Emergencies****35. MEDICAL EMERGENCY RESPONSE — ALL TYPES**

Emergency Type	Immediate Response	Call 911 if...
Unresponsive / unconscious youth	Tap shoulder and shout; check for normal breathing; if no response and no breathing: begin CPR immediately; send another staff to get the AED	Always — immediately
Cardiac arrest (no pulse, no breathing)	Call 911; begin CPR (30 compressions to 2 breaths; compressions only if untrained in rescue breaths); use AED immediately when it arrives; do not stop until paramedics arrive	Always — immediately
Choking — conscious youth	5 back blows between shoulder blades; 5 abdominal thrusts (Heimlich); alternate until expelled or youth becomes unconscious; if unconscious: begin CPR	If choking does not resolve within 1 minute
Seizure	Protect from injury; clear the area; do not restrain; do not put anything in the mouth; time the seizure; recovery position when it stops; stay with the youth	First seizure; lasts more than 5 minutes; no recovery between seizures; injury during seizure; youth has no known seizure history
Anaphylaxis / severe allergic reaction	Administer EpiPen immediately if prescribed; keep youth lying flat with legs elevated unless breathing is easier sitting; second EpiPen after 5–15 min if no improvement	Always, simultaneously with EpiPen; anaphylaxis always requires hospital assessment
Severe bleeding	Apply direct pressure with a clean cloth; maintain constant pressure; do not remove the cloth; add more cloths on top if soaked through; elevate the limb; wear gloves; tourniquet above the wound for life-threatening limb bleeding	If bleeding is severe or does not slow significantly with pressure in 5 minutes
Burns — thermal	Cool with cool running water for minimum 20 minutes; do not use ice; remove clothing from burn area unless stuck; cover loosely with a clean non-fluffy bandage; do not apply butter, cream, or toothpaste	Large area burns; burns to face, hands, feet, genitals, joints; all burns in children under 5; any full-thickness burn
Head injury	Do not move the youth if spinal injury is possible; keep them still; apply ice pack to reduce swelling for minor bumps only; monitor for warning signs	Loss of consciousness (any duration); confusion; severe headache; repeated vomiting; unequal pupils; inability to walk normally; any penetrating head injury



Hypoglycemia (low blood sugar) — conscious youth	15 grams of fast-acting carbohydrate: 3–4 glucose tablets; 175 mL juice or regular soda; 15 mL honey; wait 15 minutes; recheck blood sugar; repeat if still symptomatic; follow the youth's diabetes action plan	If youth becomes unconscious or unable to swallow: do not give anything by mouth; call 911 immediately; recovery position
Stroke	F.A.S.T: Face drooping; Arm weakness; Speech difficulty; Time to call 911	Always — immediately; stroke is time-critical
Diabetic ketoacidosis (DKA)	High blood sugar; fruity breath; nausea; abdominal pain; confusion; follow youth's diabetes action plan; call prescriber	If youth is confused, vomiting, or breathing rapidly: call 911

36. OVERDOSE AND POISONING

Scenario	Immediate Response
Suspected medication overdose — any medication	Call 911 immediately; call BC Poison Control (1-800-567-8911) simultaneously; bring the medication container to the paramedics; do not induce vomiting unless Poison Control specifically directs; stay with the youth; monitor breathing; recovery position if unconscious and breathing; notify PC and PM
Opioid overdose (unresponsive; slow/stopped breathing; blue lips; pinpoint pupils)	Call 911; administer Naloxone: insert nozzle into one nostril; press plunger firmly; repeat in other nostril after 2–3 minutes if no response; rescue breathing if not breathing; recovery position if breathing; Naloxone effect wears off in 30–90 minutes — youth must go to hospital regardless of apparent recovery
Alcohol intoxication	Call 911 if: loss of consciousness; difficulty breathing; seizure; blue/grey skin. Recovery position for unconscious youth who are breathing; do not leave alone; monitor every 15 minutes; do not give food or fluids; notify PC
Intentional poisoning or self-harm by ingestion	Call 911 and Poison Control simultaneously; treat as both a medical emergency and a mental health emergency; CI Report; MCFD notification same day; BSP and Safety Plan review
Unknown substance ingested	Call Poison Control immediately (1-800-567-8911); provide all information you have (substance, amount, time); follow their instructions exactly; call 911 if directed or if the youth has any symptoms
Chemical ingestion (household products, cleaning agents)	Call Poison Control immediately; call 911 if any respiratory symptoms or loss of consciousness; do not induce vomiting for corrosive substances; if skin or eye contact also: flush with large amounts of water

37. MENTAL HEALTH AND PSYCHIATRIC CRISIS

See also Behaviour Support Manual (ECC-BSP-2026-001) Section 22. All mental health crises are responded to with the same urgency as a medical emergency.

Crisis Type	Response
Active suicidal ideation with plan or access to means	Stay with the youth; call 911 if the youth has an immediate plan and the means are accessible; call the on-call supervisor and PC simultaneously; remove access to means if safely possible (do not physically struggle with the youth for a weapon — create space and call 911); follow the youth's Safety Plan; BC Crisis Line 1-800-784-2433
Active suicide attempt	Call 911 immediately; apply first aid for any physical injury; stay with the youth; follow paramedic direction; notify PC; notify MCFD within 2 hours; CI Report
Acute psychosis (delusions, hallucinations, disorganized behaviour)	Ensure youth is safe; call the on-call supervisor immediately; reduce stimulation (quiet environment, calm voice, fewer people); do not argue with delusions; call 911 if safety risk to the youth or others cannot be managed
Severe dissociation — youth is unresponsive to verbal contact	Do not grab or startle; stay nearby; calm, quiet presence; use the youth's name softly; call the on-call supervisor; do not leave the youth alone; call 911 if dissociation does not begin to resolve and youth cannot be safely monitored
Manic episode — extreme agitation, grandiosity, no sleep	Reduce stimulation; call the on-call supervisor; contact prescriber for medication guidance; call 911 if the youth's behaviour poses a safety risk to themselves or others
Catatonia — unresponsive, rigid, or waxy flexibility	Call 911; this is a psychiatric emergency requiring medical assessment; do not leave the youth; do not try to force movement

38. COMMUNICABLE DISEASE OUTBREAK

Outbreak Response Step	Action and Authority
1. Identify the outbreak	Two or more youth or staff with the same illness symptoms within a 48-hour period; PC notified; PC notifies PM; assess whether this meets the BC Public Health Act threshold for notification to the Medical Health Officer
2. Consult Public Health	Call HealthLink BC (8-1-1) or the local health authority for outbreak



	guidance; a notifiable disease must be reported by the physician or NP, but the program should coordinate with public health on outbreak management and containment measures
3. Infection control measures	Increase hand hygiene frequency (soap and water minimum 20 seconds); increase cleaning of high-touch surfaces (minimum twice daily with appropriate disinfectant); PPE (gloves, masks) for direct care of ill youth; cohort ill and well youth where possible
4. Ill youth management	Ill youth remain in their room where possible; separate bathroom if available; separate eating utensils and laundry; staff caring for ill youth take full PPE precautions; contact prescriber if medical assessment is needed
5. Ill staff stay home	Non-negotiable during a communicable disease outbreak; activate Emergency Staffing protocol (Section 46)
6. MCFD notification	PC notifies MCFD social workers of any significant outbreak affecting youth in care; PM coordinates with MCFD on any required service changes or suspension of group activities
7. Documentation	All illness dates, symptoms, and response measures documented; report to PM and retained in the program file

39. PANDEMIC AND PROLONGED PUBLIC HEALTH EMERGENCY

Pandemic Response Element	Policy and Standard
Compliance with Provincial Health Officer (PHO) directives	All PHO orders are mandatory; program operations adjust immediately upon any PHO directive affecting residential care; any PHO directive is implemented on the same day it is received; PM communicates all PHO directives to staff before their next shift
Staff daily health screening	Before any shift entry during an active PHO-directed screening requirement: health screening completed and documented; staff with any declared symptoms of the designated communicable disease do not enter the program
PPE supply management	Minimum 4-week supply of required PPE maintained at all times during a public health emergency; PC monitors stock and restocks when supply falls below 2 weeks; PM approves emergency procurement
Vaccination compliance	Programs follow all PHO directives regarding vaccination; vaccination status tracked per PHO requirements; PM confirms compliance; exemptions documented and managed per HR Policy Binder
Visitor restrictions	PHO-directed visitor restrictions implemented immediately; youth's right to family contact maintained through video calling or other means when in-person visits are restricted; any restriction documented with the PHO



	directive as authority; RCY and Ombudsperson access is never restricted under any PHO directive
Pandemic Service Continuity Plan	PM develops a written Pandemic Service Continuity Plan within 72 hours of any provincial public health emergency declaration that affects residential care; plan submitted to ED and MCFD for approval; reviewed and updated at minimum weekly during the emergency period
Documentation requirements	All PHO compliance measures, service changes, and pandemic-related incidents documented in the program file; reported to MCFD as required by the Service Agreement

40. CHEMICAL, BIOLOGICAL, RADIOLOGICAL, NUCLEAR (CBRN) EVENTS

CBRN events are rare but require immediate, specific responses. In the event of any suspected CBRN threat or incident, the first and most important action is to call 911 and do exactly what emergency services direct. Do not attempt to assess or manage a CBRN incident without emergency services.

CBRN Scenario	Immediate Response
Unknown substance delivered to or found at the facility (suspicious powder, liquid, package)	Do not touch or disturb the substance; isolate the area; wash hands thoroughly if skin contact occurred; call 911 and describe the substance and circumstances; do not use ventilation systems in the affected area; evacuate if directed by police
Suspicious odour or taste — possible chemical release	Do not investigate; evacuate the affected area immediately; call 911 from a safe distance; report direction from which the odour is coming; shelter in place or evacuate per direction of emergency services
External CBRN incident — transportation accident; industrial release	Shelter in place unless directed to evacuate; close all windows and doors; turn off HVAC; seal gaps with tape or towels if visible contamination is nearby; follow direction of emergency services and Emergency Management BC
Nuclear event or radiological emergency	Shelter in place; close all windows and doors; turn off HVAC; go to the most interior part of the facility away from exterior walls; follow direction of emergency services and the Canadian Nuclear Safety Commission; do not go outside unless directed

PART 8 — Category 6: Youth-Specific Emergencies

41. MISSING YOUTH

Phase	Actions	Timeline
Phase 1 — Prudent Parent Search	Immediate facility search (all rooms, yard, basement, common areas); contact known friends and locations by phone; check social media; check whether youth's medications and personal items are present (indicates planned vs unplanned departure)	Begins immediately on discovery; maximum 30 minutes
Phase 2 — MCFD and Police Notification	Call MCFD Provincial Centralized Screening (1-800-663-9122) or the youth's MCFD social worker immediately; simultaneously call RCMP or local police; provide: legal name; age; physical description and photo; last known location and time; known associates; destinations; risk factors (suicide risk, substance use, medical needs, exploitation risk); medications	At 30 minutes OR immediately if the youth is at high risk
Phase 3 — PC and PM Notification	PC notifies PM immediately upon Phase 2 activation; PM notifies ED if missing more than 2 hours or youth is at high risk	Concurrent with Phase 2
Phase 4 — Ongoing Coordination	Document all steps in chronological log; cooperate fully with police; continue monitoring known contacts; do not disclose youth's care status to any unauthorized person; update MCFD every 2 hours	Continuously
Phase 5 — Youth Returns	Warm welcome; physical health check; call MCFD to confirm return; CI Report; non-punitive debrief when regulated; BSP review within 5 business days; safety plan updated	Immediately on return

⚠ NEVER disclose to any person — including family members — that a specific youth is in government care, their location, or their status as a missing person without explicit MCFD direction. Youth in care have the right to privacy about their placement status.

42. ABDUCTION AND UNAUTHORIZED REMOVAL

Situation	Immediate Response
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Youth forcibly removed by an identified or unknown person	Call 911 immediately; call MCFD simultaneously; provide description of the youth; description of the abductor; vehicle description (make, colour, licence plate if seen); direction of travel; do not pursue; stay on the line with 911; notify PM
Youth leaves willingly with an unauthorized person	Call 911 and MCFD simultaneously; provide all descriptions; document exactly what you observed; do not physically intervene if it would put you or the youth at greater risk; stay on the line with 911
Attempted abduction — person tries to take youth but the youth remains	Call 911 immediately; implement lockdown; secure the youth inside; document everything; do not let the person inside for any reason; notify PC and PM
Non-custodial parent or family member attempts to remove youth from care	MCFD direction controls custody of youth in care; the program does not release youth to any family member without explicit MCFD authorization; call 911 if the family member is threatening or attempts to force entry; call MCFD immediately; notify PM

43. YOUTH-PERPETRATED VIOLENCE OR WEAPONS

Situation	Response
Youth is in possession of a weapon (knife, improvised weapon, etc.)	Do not attempt to take the weapon by force; create distance between the youth and others; call 911 immediately if anyone is in danger; call the on-call supervisor; use verbal de-escalation from a safe distance; do not corner the youth; do not put yourself between the youth and an exit; notify PC
Youth has injured another person with a weapon	Call 911 immediately; provide first aid to the injured person if it is safe to do so; separate all other youth from the situation; do not restrain the armed youth; notify PC and PM; CI Report; MCFD notification same day
Youth makes a credible verbal threat of violence with a weapon	Take all threats seriously; notify the on-call supervisor immediately; consider whether lockdown is required; document exact words; assess whether police should be called; notify PC and PM
Firearm discovered on the property	Do not touch; call 911 immediately; move all youth and staff away from the area; notify PC and PM; wait for police to manage

44. DISCLOSURE OF ABUSE OR NEGLECT DURING EMERGENCY

An emergency situation may prompt a youth to disclose abuse or neglect — by a staff member, a family member, or another person. Any disclosure made during an emergency is treated with the same seriousness as any other disclosure.



Response Step	Action
1. Listen and respond calmly	Do not express shock, disbelief, or excessive emotion; do not promise confidentiality; do not ask leading questions; listen and acknowledge
2. Report to MCFD immediately	Under CFCSA s.14, any person who has reasonable grounds to believe a child has been or is likely to be physically or emotionally harmed, sexually abused or exploited, or neglected has a personal duty to report to MCFD (1-800-663-9122). This duty is non-delegatable and cannot be satisfied by reporting to a supervisor.
3. Document the disclosure	Document the youth's exact words as accurately as possible; note the time and context; do not attempt to investigate or gather additional information
4. Notify PC and PM	After reporting to MCFD; PC and PM determine whether a CI Report is required
5. Support the youth	The youth's immediate safety and wellbeing take priority; provide supportive presence; ensure the youth is not left alone with the person named in the disclosure

45. DEATH OF A YOUTH IN CARE

Step	Action	Responsible
1. Call 911 immediately	Any unresponsive youth who is not breathing: CPR is initiated if the person is not obviously deceased and 911 is called immediately; do not assume death without medical confirmation	First staff to discover
2. Do not disturb the scene	Once death is confirmed by paramedics or a physician: do not move the person; do not clean or disturb anything in the area; paramedics and police manage the scene	Shift lead directs; all staff
3. Support other youth	Other youth in the home need immediate care and compassionate support; assign dedicated staff to each youth; honest, age-appropriate information is provided under PC guidance; no youth is left alone	Shift lead assigns; PC advises
4. Notify PC and PM simultaneously	Shift lead calls both immediately	Shift lead
5. Notify MCFD — within 1 hour	PM notifies MCFD Provincial Centralized Screening and the youth's social worker; MCFD will notify the family — do not contact the family without explicit MCFD direction	PM



6. Notify the Licensing Officer	PM notifies the Health Authority Licensing Officer	PM
7. Cooperate fully with police and coroner	A death in care is investigated by police and the BC Coroners Service; all staff are cooperative and truthful; no staff discusses the event with media or anyone outside the notification chain; direct all media to ED	All staff; PM leads
8. Secure all records	All records pertaining to the youth are secured and not altered, deleted, or unnecessarily accessed; PM provides records to police, coroner, or MCFD per legal counsel and MCFD direction	PM; legal counsel advises
9. Provide staff support	All involved staff receive immediate EFAP support; post-incident debrief within 48 hours; ongoing support; this is a traumatic event; no staff is expected to simply continue without support	PM and ED ensure resources
10. Accreditation notification	ED notifies the accreditation body per their standards if required (Accreditation Canada requires notification of sentinel events including unexpected deaths)	ED

PART 9 — Category 7: Organizational and Staffing Emergencies

46. CRITICAL STAFFING SHORTAGE

Staffing Shortage Step	Action
1. Anticipate and notify early	The moment a shift lead becomes aware of a potential shortage for an upcoming shift: notify PC immediately (minimum 4 hours in advance where possible)
2. Activate emergency staffing list	PC contacts all available casual and relief staff in order; documents every contact attempted (name, time, response)
3. PM escalation at 2 hours	If PC cannot resolve the shortage within 2 hours: PM notified immediately; PM contacts agency staff where authorized; PM may temporarily redeploy staff from other programs
4. On-call supervisor coverage	If no other resolution: the on-call supervisor attends the program in person as a last resort
5. MCFD notification — mandatory	If minimum staffing ratios cannot be maintained: PM notifies MCFD social worker and Provincial Centralized Screening immediately; this is a reportable safety event
6. Documentation	All steps taken are documented in the shift log and submitted to PM; CI Report if minimum ratios were not maintained

⚠ A staff member MAY NOT leave a shift until a qualified replacement has arrived and received a proper verbal handover. Leaving youth without sufficient staff coverage is abandonment of duty and a reportable safety incident under the WCA and MCFD SHSS, regardless of the personal circumstances that created the shortage.

47. DEATH OR INCAPACITATION OF A STAFF MEMBER DURING SHIFT

Situation	Response
Staff member medical emergency — serious	Call 911 immediately; apply first aid within your certification; notify PC immediately; arrange emergency coverage before injured staff member leaves; WorkSafeBC serious incident phone report if hospitalization results (1-888-621-7233 immediately); notify PM; WorkSafeBC Form 7 within 3 business days
Shift lead is the incapacitated person	Most senior YCW assumes shift lead role; calls PC immediately; PC attends the facility or arranges PM coverage; does not leave until



	coverage arrives
Staff member appears impaired by alcohol or drugs	Do not leave the impaired staff member on duty; call PC immediately; PC makes arrangements; document observations; PM notified same day; this is both a safety emergency and an HR matter per HR Policy Binder
Staff member dies during or immediately after a shift	Call 911; do not leave the person; begin CPR if they have no pulse and are not breathing; cooperate with police; notify PM and ED immediately; WorkSafeBC immediate phone report; JHSC notified; comprehensive support provided to all staff who witnessed or were present

48. DATA BREACH AND CYBERSECURITY EMERGENCY

Cybersecurity Incident	Response
Ransomware or malware attack on program systems	Disconnect affected devices from the network immediately (unplug Ethernet; disable WiFi); do not pay any ransom without legal counsel and ED authorization; call the IT support provider; notify PC and PM; notify ED; assess what data may have been compromised; prepare for BC OIPC notification if personal information is affected
Unauthorized access to youth personal records	Notify PM and ED immediately; assess scope of the breach; notify BC OIPC (oipc.bc.ca) if the breach poses a real risk of significant harm; notify affected youth and MCFD as directed by OIPC; document all steps taken
Loss or theft of a device containing youth information	Notify PC and PM immediately; assess what information was on the device and whether it was encrypted; notify OIPC if required; notify MCFD if youth records were on the device
Phishing attack — staff credentials compromised	Change all passwords immediately; notify IT provider; assess what systems were accessed; notify PM; notify OIPC if personal information was accessed

49. MEDIA CRISIS AND REPUTATIONAL EMERGENCY

A media crisis may arise from a serious incident at the program, a complaint that becomes public, or any event that attracts media attention. Evergreen Community Care's response must be consistent, controlled, and transparent within appropriate limits.

Media Crisis Response	Policy
No staff speaks to media	The Executive Director is the ONLY authorized spokesperson for



	Evergreen Community Care. No other staff member at any level speaks to any media outlet, confirms or denies any information, or allows filming of the facility or youth under any circumstances.
Media at the facility	Do not open the door to media crews; do not speak through windows; direct all media to contact the ED by name and number; call the ED immediately; do not allow filming of youth or the facility
Social media posts about an incident	No staff member posts, comments on, or shares any information about any incident, any youth, or any organizational matter on any platform. Screenshot and report any public social media posts about the program to PM and ED immediately.
ED communications	ED prepares all public statements with legal counsel; statements are factual, compassionate, and protective of the privacy and dignity of everyone involved; ED coordinates with MCFD communications before any public statement relating to MCFD-placed youth

50. FINANCIAL EMERGENCY AND SERVICE INTERRUPTION

Financial Emergency	Response
Sudden and significant funding disruption	ED notified immediately; ED consults Board and legal counsel; MCFD Contract Manager notified; plan for service continuity or transition developed within 24 hours; youth are never abandoned due to financial circumstances — MCFD has a legal obligation to ensure placement continuity
Key staff payroll failure	PC notifies PM immediately; PM notifies ED; ED resolves payroll failure within the pay period and no later; staff have the right to be paid on time per the Employment Standards Act
Insurance lapse or claim denied in a significant emergency	ED notified immediately; legal counsel engaged; organizational resources are prioritized for the immediate safety of youth in care regardless of insurance status



PART 10 — Post-Emergency Procedures

51. POST-EMERGENCY DEBRIEF PROTOCOL

Debrief Level	Trigger	Led By
Immediate operational debrief (within 1 hour of resolution)	Every emergency event at any classification level; every drill; every false alarm requiring evacuation	Shift lead; PC if available
Full debrief (within 48 hours)	Level 2 or Level 3 emergency; any injury to youth or staff; any event where the response departed significantly from the procedure	PC or PM
Root cause and improvement review (within 5 business days)	Level 3 emergency; any death or serious injury; any event revealing a significant gap in this plan; any accreditation finding related to emergency preparedness	PM; ED for Level 3 events

Debrief Component	Questions
What happened?	Factual chronological account; documented contemporaneously; not interpretation or judgment
What did we do well?	Genuine recognition of specific effective actions; named staff actions; what worked as designed
What did not go as planned?	Without blame or punitive intent; focus on system and procedure gaps
What does each youth need right now?	Immediate post-emergency care and therapeutic support for each youth
What do our staff need right now?	Secondary traumatic stress response assessment; EFAP offered; identify anyone needing immediate support
What do we change?	At least one corrective action with a named owner and target date; tracked in the Corrective Action Log

52. CRITICAL INCIDENT REPORTING — ALL EMERGENCY TYPES

Emergency Event	MCFD CI Category	Timeline
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Fire requiring evacuation	Category 1	Verbal same day; CI Report within 24 hours
Injury to a youth requiring medical attention	Category 1	Same day verbal; CI Report within 24 hours
Natural disaster requiring evacuation or relocation	Category 1	Same day verbal; CI Report within 24 hours
Medical emergency requiring 911	Category 1	Same day verbal; CI Report within 24 hours
Overdose or poisoning — youth	Category 1	Same day verbal; CI Report within 24 hours
Missing youth	Category 1	Verbal at 30 minutes; CI Report on resolution
Abduction or unauthorized removal	Category 1	Simultaneous with 911; CI Report within 24 hours
Lockdown activation	Category 1	Same day verbal; CI Report within 24 hours
Youth-perpetrated violence with weapon	Category 1	Same day verbal; CI Report within 24 hours
Death of a youth in care	Category 1	Immediate; MCFD leads documentation process
Workplace violence resulting in staff injury	Category 1	Same day; WorkSafeBC Form 7 also required
Communicable disease outbreak	Category 2 minimum	Next business day; public health may require additional reporting
Power failure exceeding 12 hours	Category 2	Next business day
Property damage exceeding \$500	Category 2	Next business day
Pandemic or public health emergency affecting operations	Category 2 minimum; escalate per PHO direction	Per PHO requirements
Data breach	Category 1 or 2 depending on scope	PM advises; OIPC may



affecting youth records	require separate notification
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WorkSafeBC reporting obligations are separate and additional to MCFD CI reporting. Any workplace injury requiring medical treatment beyond first aid: Form 7 within 3 business days (WCA s.150). Any serious incident (fracture, hospitalization, fatality, collapse): immediate phone report to WorkSafeBC (1-888-621-7233) before the scene is disturbed (WCA s.172).

53. TRAUMA-INFORMED RECOVERY FOR YOUTH AND STAFF

53.1 Youth Recovery — Trauma-Informed Principles

- Youth who experience or witness an emergency may exhibit: increased anxiety, hypervigilance, regression, nightmares, aggression, withdrawal, or somatic complaints. These are normal trauma responses, not behaviour problems.
- Increased relational presence and consistency are the most important therapeutic interventions following any emergency
- PC contacts the MCFD social worker within 24 hours to discuss any additional clinical support needed; PM requests clinical consultation if any youth has a significant post-emergency trauma response
- BSP is reviewed and updated within 5 business days of any Level 2 or 3 emergency to reflect new trauma response patterns
- Youth are given honest, age-appropriate information about what happened; they are not shielded from all information but are protected from traumatic detail; PC advises on age-appropriate communication
- Culturally safe recovery: Indigenous youth have access to cultural and spiritual support; LGBTQ2S+ youth have their identities affirmed; all youth have access to mental health services in their preferred language

53.2 Staff Recovery — Secondary Traumatic Stress

- Responding to emergencies is a significant occupational stressor; secondary traumatic stress, vicarious trauma, and compassion fatigue are recognized occupational health risks in residential care
- EFAP is actively and personally offered to all staff involved in any Level 2 or 3 emergency within 24 hours — not mentioned generically in a team meeting but offered directly to each staff member
- No staff member is expected to return to full duty in the immediate aftermath of a significant emergency without check-in and support
- Modified duties are available for any staff member experiencing secondary traumatic stress that affects their capacity to work safely
- PM monitors all staff involved in significant emergency events for secondary traumatic stress for a minimum of 4 weeks post-event; supervision sessions explicitly address wellbeing



54. RETURN TO NORMAL OPERATIONS

Return to Operations Step	Requirement
Physical safety confirmed	Relevant authorities (fire department, public health, structural engineer, utility company) have cleared the facility; PM documents the clearance and date
Medications and records reconciled	All medications accounted for; any missed doses managed per prescriber direction; all records recovered or reconstructed; CDS count reconciled
Youth readiness assessed	Each youth's readiness to return to normal routine assessed; additional supports in place before return to full routine
Staff readiness confirmed	All staff scheduled for return to normal operations have been debriefed; no staff member returns to duty while in active secondary traumatic stress crisis
MCFD and licensing notified	PM notifies MCFD and the Licensing Officer of the return to normal operations
Accreditation documentation updated	All emergency-related documentation (drill records, incident reports, corrective actions) filed and available for accreditation review
Corrective actions initiated	All corrective actions identified in the post-emergency debrief assigned, logged, and in progress; PM reviews at next supervision
Lessons learned documented	A written lessons-learned summary is added to the Emergency Preparedness file and informs the next annual review



PART 11 — Training, Monthly Drills, and Annual Review

55. MANDATORY EMERGENCY TRAINING REQUIREMENTS

All emergency preparedness training is completed before a staff member works their first unsupervised shift. Training records are maintained in the Training Compliance Matrix and are available as accreditation evidence at all times.

Training	Content Summary	Initial Deadline	Renewal
Facility emergency orientation	Physical walkthrough: all exits; assembly point; fire extinguishers; first aid kit; AED; emergency contacts; evacuation plan; Emergency Documentation Package location; their specific role in each emergency	Day 1 — before first shift; every new program assignment	Annual review at orientation anniversary
Fire safety — RACE and PASS	RACE procedure memorized; evacuation procedure; fire extinguisher types; practical PASS technique	Day 1 — before first shift	Reviewed annually at July team meeting; practical demonstration every 2 years
Fire drill participation	Full evacuation drill to assembly point; timed	Participation in next scheduled drill following hire	Every drill — quarterly minimum
First Aid / CPR Level C	CPR adult/child/infant; AED operation; choking response; bleeding control; burns; shock response	Before first shift or within 30 days	Per certification cycle (2 years typical)
OFA Level 2 (designated FAA per shift)	Full occupational first aid curriculum	Before first duty as designated FAA	Per OFA 2 cycle
Earthquake — Drop, Cover, Hold On	Procedure for staff; how to guide youth; post-earthquake assessment; facility shutoffs	Day 1 orientation	Annual earthquake drill
Lockdown / intruder / bomb threat	Lockdown procedure; communication with police; keeping youth calm; bomb threat call procedure	Day 1 orientation	Annual functional drill
Missing youth protocol	Phase 1 search; Phase 2 notification; documentation; return procedure	Day 1 orientation; within 30 days complete review	Annual; also reviewed when protocol is updated
Workplace violence prevention	Risk factors; de-escalation; calling for backup; reporting; Post-incident response	Before first shift	Annual



Communicable disease / infection control	Standard precautions; outbreak response; PPE use; reporting requirements	Within 30 days	Annual
Naloxone (where applicable)	Signs of opioid overdose; nasal spray technique; calling 911; recovery position	Before first shift where opioids are in use	Annual
EpiPen (where applicable)	Signs of anaphylaxis; EpiPen administration; second dose; calling 911	Before first shift where youth with anaphylaxis risk is in residence	Annual
WHMIS / GHS	Hazard symbols; SDS location and use; PPE selection; spill response; chemical storage	Before first shift	Every 3 years; when new hazardous products are introduced
This Emergency Preparedness Plan — full read and sign Appendix L	All 12 parts and all appendices; emphasis on emergency contacts and their specific role	Before first shift	Upon each annual update (July)

56. MONTHLY DRILL AND TRAINING CALENDAR — 12-MONTH SCHEDULE

The Program Coordinator is responsible for all monthly training and drill activities. All activities are documented on Appendix H. PM reviews and signs off by the 20th of each month. These activities are maintained as accreditation evidence.

Month	MANDATORY Activities	Drill / Training Activity
JANUARY	Monthly safety inspection (Appendix I); 72-hour supply inventory review; emergency contact directory quarterly review; training compliance matrix updated; all January First Aid and NVC I expiries confirmed current	TABLETOP DRILL: Earthquake scenario. Staff work through: Drop/Cover/Hold On; post-earthquake assessment; gas shutoff; 72-hour supply activation; notification chain. Duration: 45 minutes. Facilitator: PC.
FEBRUARY	Monthly safety inspection; smoke and CO detector tests (log results); fire extinguisher visual inspection; EpiPen and Naloxone expiry checks; FIRE DRILL (Quarter 1 — announced) — full evacuation to assembly point; timed; documented on Appendix G	POST-DRILL DEBRIEF after the fire drill — all staff who participated; what went well; what needs improvement; corrective actions assigned. New staff complete fire evacuation walkthrough if not done at hire.
MARCH	Monthly safety inspection; 72-hour supply check and restock; AED pads and battery expiry check; emergency	TABLETOP DRILL: Critical staffing shortage scenario. Who do you call? What if they are unavailable? At what point do you call MCFD?



	lighting test; hot water temperature check	Who covers until the shift lead can leave? Duration: 30 minutes. Facilitator: PC.
APRIL	Monthly safety inspection; smoke and CO detector tests; FIRE DRILL (Quarter 2 — announced); youth Emergency Profiles reviewed and updated for all current residents; go-bags checked and restocked	FUNCTIONAL DRILL: Lockdown procedure. Walk through the lockdown procedure with all current staff: who announces lockdown; how to secure exterior doors and windows; where to move youth; how to communicate with police. Duration: 30 minutes. Facilitator: PC or PM.
MAY	Monthly safety inspection; 72-hour supply check; training compliance matrix updated; all May First Aid and training expiries confirmed; emergency contact directory confirmed current	TABLETOP DRILL: Missing youth scenario. Walk through Phase 1 (30-minute search) and Phase 2 (MCFD and police notification) in detail; practice the phone calls to MCFD and police; review what information is required; review documentation requirements. Duration: 45 minutes. Facilitator: PC.
JUNE	Monthly safety inspection; smoke and CO detector tests; fire extinguisher visual inspection; emergency lighting test; FIRE DRILL (Quarter 3 — announced); gas shutoff valve location confirmed with all current staff	TABLETOP DRILL: Medical emergency scenario — youth is found unconscious. Walk through: first response; calling 911; CPR; AED use; notification chain; documentation; MCFD notification. Duration: 30 minutes. Facilitator: PC.
JULY — ANNUAL REVIEW MONTH	Monthly safety inspection; Annual Emergency Preparedness Review (Section 58; Appendix K); all emergency procedures reviewed and updated; all Appendices reviewed and updated; all staff sign updated Appendix L acknowledgement; training compliance review for all staff — any gaps addressed before end of month	ANNUAL UNANNOUNCED FIRE DRILL — target evening, night, or weekend shift; PM or PC observes and evaluates; timed; full documentation on Appendix G; post-drill debrief. OR: Full-scale multi-emergency exercise for experienced programs.
AUGUST	Monthly safety inspection; smoke and CO detector tests; FIRE DRILL (Quarter 4 — announced); AED check; 72-hour supply restock; seasonal review (add summer heat supplies if needed)	TABLETOP DRILL: Communicable disease outbreak scenario. Staff work through: identifying an outbreak; infection control measures; cohorting ill youth; notifying public health; ill staff stays home — who covers?; MCFD notification. Duration: 30 minutes. Facilitator: PC.
SEPTEMBER	Monthly safety inspection; 72-hour supply autumn review (begin adding winter supplies — extra blankets, warm clothing, hand warmers); emergency contact directory quarterly review; wildfire and extreme heat season debrief	TABLETOP DRILL: Overdose response scenario. Staff work through: recognizing opioid overdose signs; Naloxone administration; calling 911; recovery position; what information to give paramedics; what to tell the youth; MCFD notification and CI Report. Duration: 30 minutes. Facilitator: PC.
OCTOBER	Monthly safety inspection; CO detector test (especially important as heating season begins); fire extinguisher visual	FUNCTIONAL DRILL: EpiPen administration on a training auto-injector — all direct-care staff demonstrate correct technique; review signs of



	inspection; winter 72-hour supply additions completed; heating system functionality confirmed; hot water temperature check	anaphylaxis; review when to administer the second dose; review the 911 call. Duration: 20 minutes per staff member. Facilitator: PC.
NOVEMBER	Monthly safety inspection; smoke and CO detector tests; emergency lighting test; winter preparedness check — outdoor pathways safe, 72-hour winter supplies complete; all November training expiries confirmed current	TABLETOP DRILL: Bomb threat scenario. Walk through: how to handle a threatening phone call; what information to gather; calling 911; evacuation decision; cooperating with police. Duration: 30 minutes. Facilitator: PC. AND: Drop, Cover, Hold On earthquake practice —5-minute physical practice with all staff and youth.
DECEMBER	Monthly safety inspection; year-end training compliance review — all gaps identified and addressed before end of month; all Youth Emergency Profiles confirmed current for all current residents; December fire extinguisher visual inspection (holiday decoration season)	ALL-STAFF EMERGENCY PREPAREDNESS YEAR-END REVIEW MEETING: Led by PM. Review all drills conducted in the past year; review all actual emergency events and lessons learned; preview the training calendar for the coming year; recognize staff achievements in emergency preparedness. Duration: 60 minutes.

57. DRILL PLANNING, CONDUCT, AND EVALUATION STANDARDS

57.1 Drill Types and Accreditation Requirements

Drill Type	Description and Accreditation Alignment	Required Frequency
Announced evacuation drill (fire)	Full evacuation to assembly point; all staff and youth who are present participate; timed from alarm to all persons at assembly point; documented on Appendix G. Required by BC Fire Code and CARF 1.J.3.	Quarterly minimum (4 per year)
Unannounced evacuation drill	No advance notice; simulates a real emergency; conducted at a non-peak time; PM or PC evaluates; documented on Appendix G. Required by BC Fire Code and Accreditation Canada.	Annual minimum
Tabletop exercise	Scenario-based discussion drill; staff work through the response to a specific emergency scenario without physically evacuating; excellent for complex scenarios. Required by CARF 1.J.2.	Monthly recommended; see Section 56 calendar
Functional drill	A specific emergency component is physically practised (lockdown; Naloxone administration; EpiPen technique; Drop/Cover/Hold On). CARF 1.J.2 and Accreditation Canada.	As needed; minimum 3 per year on the calendar



Full-scale exercise	Realistic simulation of a major emergency involving all staff and potentially external agencies; highest resource investment; greatest learning value. CARF 1.J.2.	Every 3 years recommended; coordinates with PM and ED
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57.2 Drill Evaluation Framework

Evaluation Dimension	Questions and Standard
Time	What was the total evacuation time? From alarm to all persons at assembly point? Is it better or worse than the previous drill? Target: improving trend over time
Completeness	Was every youth and staff member accounted for? Was anyone missed or delayed? Was the Emergency Documentation Package retrieved?
Procedure adherence	Did staff follow the procedure in this plan? Were any steps skipped or incorrectly sequenced? Were any improvised steps used (and if so, are they improvements that should be incorporated)?
Youth response	How did each youth respond? Were any youth particularly distressed or at risk? Does any youth's response reveal a gap in their Emergency Profile?
Communication	Was communication between staff clear, timely, and effective? Did everyone know their role? Were any coordination gaps identified?
Equipment	Was all emergency equipment accessible and functional? Were there any equipment failures or missing items?
Documentation	Was the drill documented immediately after? Does the record capture the evaluation findings accurately?
Accreditation evidence	Is the documentation complete and ready for accreditation review? Does it demonstrate compliance with the applicable standards?

58. ANNUAL EMERGENCY PREPAREDNESS REVIEW

The Annual Emergency Preparedness Review is conducted each July by the Program Manager using Appendix K. It covers the full emergency preparedness program for the prior 12 months. The review is submitted to the ED and is part of the Annual Program Review (MCFD Standard H.2) and the accreditation self-assessment.



58.1 Quarterly Review of the Emergency Preparedness Manual

In addition to the full Annual Review, this Emergency Preparedness Policy and Plan (Policy Number ECC-EP-2026-002) undergoes a structured Quarterly Review. The quarterly review is a lighter-weight check between annual reviews to confirm the plan remains current, accurate, and operationally sound. It is conducted by the Program Coordinator and reviewed by the Program Manager.

Quarter	Quarterly Review Window	Quarterly Review Focus
Q1	January (covers October–December)	Confirm Emergency Contact Directory (Appendix B) is current; confirm all Q4 fire drill and tabletop drill records are complete (Appendices G, H); confirm winter 72-hour supplies are stocked; review any Level 2 or 3 emergencies from the prior quarter and confirm corrective actions are tracked
Q2	April (covers January–March)	Confirm Emergency Contact Directory is current; confirm all Q1 drill records are complete; confirm Youth Emergency Profiles (Appendix F) are current for all residents; review training compliance for the quarter; confirm any policy number or version references are consistent throughout the document
Q3	July (combined with the Annual Review, Section 58 above)	The Q3 quarterly check is absorbed into the full Annual Emergency Preparedness Review (Appendix K); no separate Q3 quarterly form is required
Q4	October (covers July–September)	Confirm Emergency Contact Directory is current; confirm all Q3 drill records are complete; confirm 72-hour supplies have been switched to winter readiness; review any accreditation findings from the year and confirm they are addressed; confirm the alternate facility



		agreement remains valid
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Quarterly Review Requirement	Standard
Who conducts the review	Program Coordinator conducts the review; Program Manager reviews and signs off within 5 business days
What is reviewed	Emergency Contact Directory currency; drill and training compliance for the quarter; 72-hour supply seasonal readiness; Youth Emergency Profile currency; outstanding corrective actions from the prior quarter; any legislative, accreditation, or MCFD Standard changes that may affect this plan
Documentation	A short Quarterly Review Memo (1 page) is completed and filed in the Emergency Preparedness binder; it references this plan by Policy Number ECC-EP-2026-002 and confirms the version currently in effect
Escalation	Any material gap identified in a quarterly review is escalated to the PM immediately; significant gaps may trigger an out-of-cycle update to this plan rather than waiting for the next Annual Review
Relationship to the Annual Review	The four quarterly reviews and the Annual Review together form a continuous review cycle; nothing in this plan goes more than approximately 90 days without some level of confirmation that it remains current

Annual Review Component	Content
Drill and exercise log review	Were all required drills conducted? Were the required types completed? Were deficiencies identified and corrected? Were Appendix G and H forms completed for every drill?
Actual emergency incident review	Were there any actual emergency events? How were they managed? What were the outcomes? What lessons were learned? Were CI Reports submitted on time?
Training compliance review	Are all staff current on all required emergency preparedness training? Are records complete? Are there any gaps to address in the coming year?
Equipment and facility review	All emergency equipment current and functional? All monthly inspections completed for all 12 months? Any equipment requiring upgrade or replacement?
Emergency Documentation Package review	Youth Emergency Profiles current for all current residents? Emergency Contact Directory confirmed current? Alternate facility agreement

	confirmed?
72-hour supply review	Supplies adequate, complete, and not expired? Quantities sufficient for current resident and staff count?
Procedure review	Are any emergency procedures in this plan out of date? Any changes in BC legislation, MCFD Standards, OHS Regulation, or accreditation standards that require procedure updates?
Accreditation readiness	Is emergency preparedness documentation complete and organized for accreditation review? Any accreditation findings from the prior year relating to emergency preparedness that require follow-up?
Coming year plan	Updated drill schedule; new training needs identified; facility improvements required; budget requirements for emergency preparedness
Sign-off and distribution	PM signs and dates; ED reviews and signs; submitted to Board as part of Annual Program Review; copy filed in Emergency Preparedness binder; copy to MCFD as part of program report

59. ACCREDITATION SELF-ASSESSMENT — EMERGENCY PREPAREDNESS

This self-assessment is completed as part of the Annual Emergency Preparedness Review. It maps each accreditation standard to the evidence available in this program. It is used to identify gaps before any accreditation visit.

Standard	Requirement Summary	Evidence Available	Status
Accreditation Canada — Emergency Preparedness (1.4)	Written plan; trained staff; regular drills; annual review	This plan; Appendix G drill records; Appendix L acknowledgements; Appendix K annual review	
Accreditation Canada — Environmental Safety (1.3)	Safe physical environment; inspections documented; hazards controlled	Appendix I monthly inspections; equipment records	
Accreditation Canada — Safety Culture (1.5)	Near-misses and incidents reported; corrective action tracked	Corrective Action Log; post-emergency debrief records (Appendix J)	
Accreditation Canada — Infection Control ROP	Written infection control program; outbreak response; staff competency	Sections 38, 39; staff training records	
Accreditation Canada — Workplace Violence ROP	Written violence prevention program; staff trained; incidents	Section 33; OHS Program Binder; training records	



	reported		
CARF 1.J.1 — Environmental Safety	Regular inspections; hazard identification and correction	Appendix I monthly inspections; corrective action log	
CARF 1.J.2 — Emergency Preparedness	Written procedures; trained staff; drills documented; annual review	This plan; all appendices; drill records; annual review	
CARF 1.J.3 — Fire Safety	Fire procedures; regular drills; equipment maintained; plans posted	Section 13–16; Appendix G; equipment records	
CARF 1.J.4 — Infection Control	Outbreak response; communicable disease procedures	Sections 38, 39	
CARF 1.J.5 — Security	Security procedures; staff trained	Sections 30–34	
MCFD SHSS Standard F.4	Safety and emergency practices; drills; staff trained; youth information maintained	All Parts 3–11; Appendix F youth profiles; Appendix G drill records	
BC OHS Regulation Part 4	Written emergency procedures; workers trained; drills at regular intervals	This plan; training records; drill records	
BC Fire Code — Division B Part 2	Fire safety plan; quarterly drills; annual unannounced drill; equipment maintained	Sections 13–16; Appendix G; equipment records	



PART 12 — Appendices



APPENDIX Appendix A — ALL EMERGENCIES — QUICK REFERENCE CARD

Print and laminate. Post in the shift communication area and in the staff area. Every staff member is familiar with this card. It does not replace the full plan — it is a shift-accessible quick reference.

Emergency	Immediate First Action	Call
FIRE	RACE: Rescue — Alarm — Contain — Evacuate. Pull the alarm. Evacuate to assembly point.	911 immediately
EARTHQUAKE	Drop, Cover, Hold On. Stay inside. After shaking stops: account for all persons; check for hazards.	911 if injuries or fire
GAS LEAK	Do not switch anything on or off. Evacuate immediately. Call from outside.	FortisBC 1-800-663-9911; 911 if injuries
POWER FAILURE	Emergency lighting activates. Flashlights deployed. Monitor refrigerated medications.	BC Hydro 1-888-769-3766; PM if prolonged
FLOOD — Evacuation Order	Evacuate immediately to alternate facility with go-bags.	MCFD; PM; 911 if anyone in danger
LOCKDOWN	Lock all exterior doors. Move youth to interior rooms. Call 911. Do not open for anyone until police ALL CLEAR.	911 immediately
BOMB THREAT	Keep caller talking. Get details. Call 911 from another phone.	911 immediately; PC
CARDIAC ARREST	Call 911. CPR immediately. Get the AED.	911 immediately
OVERDOSE — OPIOID	Call 911. Naloxone. Recovery position if breathing. Rescue breathing if not breathing.	911 immediately; Poison Control 1-800-567-8911
OVERDOSE — OTHER	Call 911 and Poison Control simultaneously. Do not induce vomiting.	911 + Poison Control 1-800-567-8911
ANAPHYLAXIS	EpiPen in outer thigh (through clothing OK). Hold 10 seconds. Legs elevated. Second dose after 5–15 min if needed.	911 immediately — always
MISSING YOUTH	Facility search (30 min max). Then MCFD + police. Document every step.	MCFD 1-800-663-9122; police non-emergency
SUICIDE ATTEMPT	Call 911. First aid. Stay with the youth. Call PC.	911 immediately; MCFD within 2 hours



UNAUTHORIZED REMOVAL	Call 911. Provide descriptions. Do not pursue.	911 immediately + MCFD 1-800-663-9122
DISCLOSURE OF ABUSE	Listen calmly. Do not promise confidentiality. Report to MCFD immediately.	MCFD 1-800-663-9122 (mandatory under CFCSA s.14)
STAFFING SHORTAGE	Call PC immediately. Document all contacts. Never leave youth uncovered.	PC; PM if not resolved in 2 hours
INTRUDER	Do not open the door. Move youth to interior rooms. Call 911.	911 immediately; PC
DEATH IN CARE	Call 911. Do not disturb the scene. Support other youth. Call PC and PM.	911; then PM within minutes; MCFD within 1 hour
PANDEMIC / PHO DIRECTIVE	Follow PHO directive immediately. Staff screening. PPE. Notify PM.	PM; MCFD; follow PHO guidance
STRUCTURAL DAMAGE	Evacuate if visible damage. Do not re-enter. Call PM.	911 if anyone trapped; PM immediately
WORKPLACE VIOLENCE — EXTERNAL	Do not intervene physically. Call 911. Secure youth inside.	911 immediately; PC

EMERGENCY NUMBERS — ALWAYS KNOW THESE	
911 — all life-threatening emergencies	911
BC Poison Control (24/7)	1-800-567-8911
MCFD Provincial Centralized Screening (24/7)	1-800-663-9122
On-Call Supervisor	[See posted weekly schedule]
FortisBC Gas Emergency	1-800-663-9911
BC Hydro Outages	1-888-769-3766
WorkSafeBC Serious Incidents	1-888-621-7233



**APPENDIX Appendix B — EMERGENCY CONTACT DIRECTORY**

Print, laminate, and post at every telephone, main entrance, and shift communication area. Version date displayed prominently. Updated within 24 hours of any change. Quarterly review confirmed by PC signature.

FACILITY: _____ ADDRESS: _____ Updated: _____	NUMBER / LOCATION
911 — fire, police, ambulance (all life-threatening emergencies)	911
BC Poison Control (24/7)	1-800-567-8911
Health Link BC (nurse advice, 24/7)	8-1-1
MCFD Provincial Centralized Screening (24/7)	1-800-663-9122
BC Crisis Line (24/7)	1-800-784-2433
WorkSafeBC — serious incident reporting	1-888-621-7233
BC Emergency Management Division (24/7)	1-800-663-3456
On-Call Supervisor — see posted schedule	
Program Coordinator — [Name]	[Cell]
Program Manager — [Name]	[Cell]
Executive Director — [Name]	[Cell]
Program Pharmacy — [Name]	[Number]
On-Call Physician / NP	[Number]
Hospital Emergency — [Name, Address]	[Number]
BC Hydro — power outage	1-888-769-3766
FortisBC — gas emergency (24/7)	1-800-663-9911



Local Police — non-emergency	[Number]
Building Owner / Property Manager	[Number and after-hours]
HVAC / Heating Emergency	[Number]
Representative for Children and Youth (RCY)	1-800-476-3933
GAS SHUTOFF LOCATION	[Describe — mark on evacuation plan]
ELECTRICAL PANEL / MAIN BREAKER	[Describe]
WATER MAIN SHUTOFF	[Describe]
EVACUATION ASSEMBLY POINT	[Describe — minimum 15m from building]
ALTERNATE FACILITY FOR RELOCATION	[Name, address, contact number]

Quarterly Review — confirmed current	PC Signature / Date
Q1 (January)	
Q2 (April)	
Q3 (July — part of annual review)	
Q4 (October)	



**APPENDIX Appendix C — FACILITY EVACUATION PLAN TEMPLATE**

Complete for every floor of the facility. Post on every floor, at every exit, and in the Emergency Documentation Package. Update within 30 days of any facility change. Reviewed annually.

Facility Name and Address	
Floor Number	
Date of This Plan	
Prepared By (PC name)	
Approved By (PM name)	
Next Review Date	

EVACUATION ROUTES	Description — be specific (doors, hallways, stairwells, exterior paths)
Primary exit from this floor	
Secondary exit from this floor (if available)	
External path from exit to assembly point	
Assembly point — exact location and landmark	
Alternative assembly point (if primary is blocked)	
Time target — all persons at assembly point within	[Set per last fire drill performance; target to improve]

FIRE SAFETY EQUIPMENT LOCATIONS ON THIS FLOOR	Location
Fire alarm pull station	
Fire extinguisher	
Fire hose (if applicable)	



Emergency exit lighting	
First aid kit	
AED (if on this floor)	

ROOM / AREA	Youth Name or Note	Special Evacuation Requirement
Room [number]		
Room [number]		
Room [number]		
Bathroom	Check every time; youth may be here during alarm	
Common room		
Kitchen	Staff check and turn off heat sources during evacuation	

[FLOOR PLAN DIAGRAM — Attach a sketch or printed floor plan showing: all rooms and their numbers; primary exit routes marked with arrows; secondary exit routes marked with dashed arrows; fire extinguisher locations marked; fire alarm pull station locations; AED location; assembly point direction; any rooms with youth who need special evacuation assistance marked]



**APPENDIX Appendix D — EMERGENCY EQUIPMENT CHECKLIST**

Complete monthly as part of the Monthly Safety Inspection (Appendix I). PC signs; PM reviews within 5 business days. Retained as accreditation evidence — minimum 3 years.

Facility Name	
Inspection Date	
Inspected By (PC name and signature)	
PM Review Signature and Date	

Equipment Item	Location	Status: OK / Needs Action	Action Required
Smoke detector — Floor 1 (test result)			
Smoke detector — Floor 2			
Smoke detector — all bedrooms tested individually			
CO detector — Floor 1 (test result)			
CO detector — Floor 2			
Fire extinguisher — Floor 1 (pressure gauge in green; pin and seal intact)			
Fire extinguisher — Floor 2			
Fire extinguisher — annual professional inspection current (date)			
Emergency exit lighting — activation test all lights			
Exit signs — all illuminated and visible			
All exits clear and unobstructed			
Emergency flashlights — all			



functional with fresh batteries			
Battery-powered radio — test reception			
First aid kit — fully stocked per kit specification			
AED — pads expiry date			
AED — battery indicator shows ready			
Naloxone kit — expiry date (where applicable)			
EpiPen — expiry date (where applicable)			
Emergency contact directory — posted, current, version date visible			
Evacuation plan — posted on every floor			
Youth Emergency Profiles — current for all current residents (Appendix F)			
72-hour supply inventory — reviewed and adequate (Appendix E)			
Emergency Documentation Package — accessible to shift lead			
Hot water temperature — all taps tested (maximum 49 degrees C)			
WHMIS SDS binder — current for all chemicals in use			
Gas shutoff wrench — accessible (where applicable)			
Security — all exterior doors lock from inside; peephole or intercom functional			



**APPENDIX Appendix E — 72-HOUR EMERGENCY SUPPLY INVENTORY**

Reviewed monthly. Restock immediately when items are used or expire. Quantities calculated for all current residents plus staff on a full shift. PM approves budget annually.

Facility Name	
Number of current residents	
Number of staff on a full shift	
Total persons to supply (residents + staff)	
Review Date	
Reviewed By	

Item	Minimum Required Quantity	Current Quantity	Expiry / Condition
Bottled water (4 litres per person per day x 3 days)	[Total persons x 12 litres]		
Water purification tablets (backup)	1 per emergency kit		
Non-perishable food — 3 days of 3 meals per person	[Total persons x 9 meal portions]		
Manual can opener	1		
Paper plates, cups, cutlery	3 sets per person		
72-hour medication kit (per Medication Manual s.9)	One kit per youth; current as of last prescription change		
First aid supplies — portable backup kit	1 complete kit		
Emergency thermal / mylar blankets	1 per person + 2 extra		
Warm blankets — additional (October through April)	1 per person		
Extra warm clothing layers (October through April)	1 set per person		
Hand warmers (October through April)	2 per person		



LED flashlights with extra batteries	1 per floor + 1 extra per person		
Battery-powered or hand-crank radio	1		
Portable battery packs and charging cables	1 per active staff phone		
Nitrile gloves (medium and large)	3 pairs per person		
N95 or surgical masks	1 per person + 10 extra		
Hand sanitizer (large bottle)	2		
Toilet paper	3-day supply per person		
Garbage bags — large	10		
Wipes — disinfecting and personal hygiene	1 package each		
Duct tape and heavy-duty adhesive tape	1 roll each		
Youth Emergency Profiles — copy in waterproof container	Current set for all residents		
Emergency Contact Directory — copy	1 current copy		
Cash — small bills (PM approved amount)	[PM approved amount]		
Youth comfort and activity items	Sufficient for 3 days — per youth documentation		
Special needs supplies per youth (AAC charger, medical supplies)	Per each youth's emergency profile		
Alternate facility information — address and contact	Current copy		





APPENDIX Appendix F — YOUTH EMERGENCY PROFILE AND GO-BAG CHECKLIST

One form per youth. Updated within 30 days of any ISP change. Reviewed monthly by PC. Kept in the Emergency Documentation Package accessible to the shift lead at all times. Go-bag updated whenever medications or profile content changes.

YOUTH IDENTITY	
Legal Name	
Preferred Name and Pronouns	
Date of Birth	
Photo attached (current)	Yes / No
Room Number	
MCFD Social Worker — Name, office phone, cell	
EMERGENCY CONTACTS — authorized persons only	
Contact 1 — name, relationship, phone	
Contact 2 — name, relationship, phone	
MEDICAL ALERTS	
All known allergies — substance and reaction type	
EpiPen prescribed? Location of EpiPen?	Yes / No / Location:
Naloxone prescribed? Location?	Yes / No / Location:
Critical medical diagnoses relevant to emergency response	
Any medical equipment in use (CPAP, feeding pump, etc.)	
CURRENT MEDICATIONS	



Medications — name, dose, schedule (time-critical doses highlighted)	
Prescriber name and after-hours contact	
Pharmacy name and number	
EVACUATION AND MOBILITY	
Can this youth evacuate independently by stairs and on foot?	Yes / No / Describe limitations:
Assistance required for evacuation	
Mobility aid (wheelchair, walker, crutches)?	Yes / No / Type:
Staff assigned to assist this youth in evacuation	
COMMUNICATION DURING EMERGENCIES	
Primary language	
AAC device or communication board used?	Yes / No / Type:
Cognitive level — what instructions work during stress?	
Any communication disabilities that affect emergency response	
BEHAVIOURAL RESPONSE TO EMERGENCIES	
Known triggers in emergency situations (alarms, crowds, darkness, uniforms)	
Likely behavioural responses (elopement risk, aggression, freezing,	



dissociation)	
Effective de-escalation approaches during emergency stress	
Approaches to AVOID with this youth in emergency situations	
Is this youth a high elopement risk during emergencies?	Yes / No — if Yes: what additional measures?
CULTURAL AND IDENTITY CONSIDERATIONS	
Indigenous Nation or community affiliation	
Cultural or spiritual practices relevant to emergency response	
Gender identity and any privacy considerations during emergency sheltering	
Language preferences for communication during crisis	
GO-BAG CONTENTS — confirm each item is current and packed	
72-hour medication supply — labelled, secure, updated date:	Packed? Yes / No / Date updated:
Copy of this Emergency Profile	Packed? Yes / No
Health card and personal ID	Packed? Yes / No
Comfort items — list:	Packed? Yes / No
Changes of clothing — weather-appropriate; count:	Packed? Yes / No
Phone charger / portable battery	Packed? Yes / No
AAC device or communication board	Packed? Yes / No



Sensory or calming items — list:	Packed? Yes / No
Special medical supplies — list:	Packed? Yes / No
DATE OF LAST UPDATE	
PC SIGNATURE	
PM REVIEW SIGNATURE	



**APPENDIX Appendix G — FIRE DRILL RECORD**

Complete immediately after every fire drill. Retained as accreditation evidence — minimum 3 years. Submit to PM within 5 business days.

Facility Name	
Date of Drill	
Time of Alarm Activation	
Time All Persons Accounted for at Assembly Point	
Total Evacuation Time (from alarm to all clear at assembly point)	
Announced or Unannounced?	
Drill Type	Quarterly announced / Annual unannounced / Night drill / New staff orientation drill
Number of Youth Present	
Number of Staff Present	
PM or PC Present as Observer?	Yes / No — if Yes, name:
Were ALL persons accounted for at the assembly point?	Yes / No — if No, describe:
Was the Emergency Documentation Package retrieved?	Yes / No
Were all exits accessible and unobstructed?	Yes / No — if No, describe:
Was any youth significantly distressed?	Yes / No — if Yes, describe and note BSP implications:
Did any youth require special assistance for evacuation?	Yes / No — if Yes, describe and evaluate effectiveness:
Were any procedures not followed correctly?	Yes / No — if Yes, describe:



Were any equipment issues identified?	Yes / No — if Yes, describe:
Overall assessment	Satisfactory / Needs Improvement — explain if Needs Improvement:
Corrective actions identified	
Corrective action 1 — action, owner, target date	
Corrective action 2 — action, owner, target date	
Post-drill debrief conducted?	Yes / No Date: _____
Accreditation standard met by this drill?	BC Fire Code quarterly drill; Accreditation Canada; CARF 1.J.3; MCFD SHSS F.4
PC Signature	
PM Review Signature and Date	



**APPENDIX Appendix H — EMERGENCY DRILL RECORD — ALL TYPES**

Complete for every tabletop exercise, functional drill, or emergency exercise that is not a fire drill (fire drills use Appendix G). Retained as accreditation evidence — minimum 3 years.

Facility Name	
Date of Drill	
Type of Drill	Tabletop / Functional / Multi-hazard exercise / Other:
Emergency Scenario Practised	
Accreditation Standard Addressed by This Drill	CARF 1.J.2 / Accreditation Canada 1.4 / OHS Reg. s.4.16 / MCFD SHSS F.4 / Other:
Staff Who Participated	
Lead Facilitator (PC or PM name)	
Duration	
Learning Objectives for This Drill	
Summary of Scenario and What Was Practised	
What went well?	
What did not go as planned?	
Knowledge or procedure gaps identified	
Were any youth significantly distressed? (if applicable)	
Corrective actions identified (with owner and target date)	
Overall drill assessment	Satisfactory / Needs Improvement
Post-drill debrief conducted?	Yes / No Date: _____
PC Signature	
PM Review Signature and Date	





APPENDIX Appendix I — MONTHLY SAFETY AND EMERGENCY INSPECTION CHECKLIST

Complete by the last business day of each month. Submit to PM within 5 business days. PM reviews and signs within 5 additional business days. Retained as accreditation evidence — minimum 3 years.

Facility Name	
Month / Year	
Number of current residents at time of inspection	
PC Name and Signature	
PM Review Signature and Date	

Inspection Item	Standard	Status: OK / P1 / P2 / P3	Action / Notes
Smoke detectors — all tested by button press	Pass every test; date of test recorded		
CO detectors — all tested	Pass every test; date of test recorded		
Fire extinguishers — visual inspection (all units)	Pressure gauge green; pin and tamper seal intact; no visible damage		
Annual professional inspection — certificate on file?	Current year certificate on file		
Emergency exit lighting — activation test	All lights activate and illuminate exit routes		
Exit signs — all illuminated and visible	All illuminated; no burned-out bulbs		
All exits clear and unobstructed	No furniture, boxes, or objects blocking any exit at any time		
Emergency flashlights — all functional; batteries checked	All functional; fresh batteries		



Battery radio — test reception	Receives AM/FM and emergency broadcasts		
First aid kit — fully stocked; nothing expired	All items present and in-date per kit specification		
AED — pads expiry date confirmed	Pads not expired; within-date indicator confirmed		
AED — battery / ready indicator confirmed	Ready indicator shows green or equivalent		
Naloxone kit — expiry confirmed; accessible (where applicable)	Not expired; stored outside medication cabinet		
EpiPen — expiry confirmed; accessible (where applicable)	Not expired; stored outside medication cabinet; known to all staff		
Emergency contact directory — posted; current; version date visible	Posted at all phones and main entrance; all numbers confirmed current this quarter		
Evacuation plan — posted on every floor; arrows visible	Current version; unobstructed arrows; posted at every floor and exit		
Youth Emergency Profiles — current for all current residents	Updated within 30 days of any ISP change; reviewed this month		
Go-bags — reviewed and current (medications updated)	Each youth's go-bag reviewed; medications updated if any prescription change		
72-hour supply inventory — quantities adequate; nothing expired	All items per Appendix E; quantities for current resident and staff count		
Emergency Documentation Package — accessible to shift lead	Package location known to shift lead; contents current		



Hot water temperature — all taps tested with thermometer	Maximum 49 degrees C at all hot water taps		
WHMIS SDS binder — current for all chemicals in use	SDS for all current products; no products used without SDS		
Medication cabinet — locked; keys accounted for	Locked at all times except during administration; key log current		
Heating system — functional and maintaining safe temperature	Interior minimum 18 degrees C at all times; thermostat functional		
Gas shutoff wrench — accessible (where applicable)	Located near the shutoff valve; location known to shift leads		
Security — exterior doors lock from inside; intercom functional	All exterior locks functional; peephole or intercom operable		
Outdoor pathways and exits — safe; clear of hazards	Ice, snow, and debris removed; safe walking surface; winter treatment applied as needed		

SUMMARY OF DEFICIENCIES THIS MONTH	
Number of Priority 1 deficiencies	
Number of Priority 2 deficiencies	
Number of Priority 3 deficiencies	
All Priority 1 deficiencies resolved before end of inspection?	Yes / No — if No, describe:
Open corrective actions from	



prior months — status	
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**APPENDIX Appendix J — POST-EMERGENCY DEBRIEF FORM**

Complete for every Level 2 or Level 3 emergency event within 5 business days. Retained as accreditation evidence and in the Emergency Preparedness file. PM submits lessons learned to ED.

Facility Name	
Date of Emergency Event	
Emergency Classification (Level 1 / 2 / 3)	
Emergency Category and Type	
Brief description of the event	
Date and Facilitator of Immediate Operational Debrief (within 1 hour)	
Date and Facilitator of Full Debrief (within 48 hours)	
Staff Who Participated in Full Debrief	
CHRONOLOGICAL EVENT LOG	
Time event was identified or began	
Actions taken — in chronological order	
Time event was resolved or all persons were safe	
Total duration of emergency response	
ASSESSMENT	
What did staff do well? (specific and named)	
What did not go as planned?	
Were any procedures not followed? If so, which and why?	



Were any equipment failures or gaps identified?	
Were any youth significantly distressed?	
Post-emergency support provided to youth (describe)	
Were any staff injured?	
EFAP offered to all involved staff within 24 hours? (yes / no)	
REPORTING COMPLETED	
MCFD CI Report filed? (category; date submitted)	
WorkSafeBC report filed? (Form 7 or immediate report; date)	
Licensing Officer notified? (yes / no / not required)	
Accreditation body notified? (yes / no / not required)	
CORRECTIVE ACTIONS	
Corrective action 1 — what; owner; target date	
Corrective action 2 — what; owner; target date	
Corrective action 3 — what; owner; target date	
LESSONS LEARNED — for the annual review	
Any procedure in this plan that should be updated as a result of this event?	Yes / No — if Yes, identify the section and proposed change:
PC Signature and Date	
PM Review Signature and Date	





APPENDIX Appendix K — ANNUAL EMERGENCY PREPAREDNESS REVIEW FORM

Complete each July. Submitted to ED and included in the Annual Program Review (MCFD Standard H.2). Retained as accreditation evidence. Minimum 5-year retention.

Facility Name	
Review Year	
Program Manager Name	
Date Submitted to ED	
ED Signature and Date	

Review Area	Standard	Findings This Year	Action for Coming Year
Fire drills — 4 quarterly announced drills conducted	4 minimum; all documented on Appendix G		
Annual unannounced fire drill conducted	1 minimum; documented on Appendix G		
Monthly tabletop or functional drills — 12 month calendar	12 drills minimum; documented on Appendix H		
All required emergency training current for all staff	Per Section 55 training matrix		
Monthly safety inspections — 12 completed this year	12 inspections minimum; Appendix I		
Emergency equipment — all in-date and functional	Monthly checks; annual professional inspections		
Youth Emergency Profiles current for all residents	Updated within 30 days of any ISP change		
Go-bags current and medication kits updated	72-hour medication supply current		
72-hour supply inventory complete and current	Monthly review; Appendix E		
Emergency Contact Directory — quarterly reviews completed	4 quarterly reviews		



Alternate facility agreement confirmed current	Annual confirmation		
Actual emergencies this year — number and types	All CI Reports filed; all debriefs completed		
Corrective actions from prior year — all completed	All corrective actions from last year's review		
Lessons learned incorporated into the plan	This plan updated based on lessons learned		
Accreditation self-assessment (Section 59) completed	All standards assessed; gaps identified		
Staff acknowledgements (Appendix L) — all staff signed updated version	100% — all current staff		

SUMMARY — ANNUAL EMERGENCY PREPAREDNESS ASSESSMENT	
Overall assessment this year	Satisfactory / Needs Improvement
Material gaps or concerns identified	
Priority improvements for the coming year	
Budget requirements for emergency preparedness — coming year	
Accreditation readiness — is emergency preparedness documentation complete?	Yes / No — gaps:
This review has been submitted to the Board of Directors?	Yes / No — date submitted:





APPENDIX Appendix L — STAFF EMERGENCY PREPAREDNESS ACKNOWLEDGEMENT FORM

All staff, students, and volunteers must read and sign this acknowledgement before their first shift and upon each annual update. Retained as accreditation evidence.

I acknowledge that I have received, read, and understood the Evergreen Community Care Emergency Preparedness Policy and Plan (ECC-EP-2026-002), Version 2.0, effective July 1, 2026.	
I have completed the facility emergency orientation, including a physical walkthrough of: all exits and the evacuation route for my assigned area; the assembly point location; fire extinguisher, first aid kit, and AED locations; emergency contact directory location; Emergency Documentation Package location; my specific role during each category of emergency.	
I can describe the RACE procedure (Rescue, Alarm, Contain, Evacuate) and my role during a fire emergency without reference to this document.	
I can describe the Drop, Cover, Hold On procedure for earthquake response.	
I understand the lockdown procedure and my role if a lockdown is declared.	
I understand that every alarm is treated as real until the fire department confirms otherwise. I do not investigate before evacuating.	



I know the location of the Emergency Documentation Package and the Youth Emergency Profiles and can retrieve them without assistance.	
I understand that I may not leave a shift until a qualified replacement has arrived and received a verbal handover, regardless of any personal circumstances.	
I understand that in an emergency, the shift lead is the on-site Incident Commander and I follow their direction; if the shift lead is incapacitated, I call the PC immediately.	
I understand my mandatory reporting obligation under CFCSA s.14 — if I have reason to believe a child has been or is likely to be harmed, I report to MCFD (1-800-663-9122) personally, regardless of any instruction from my supervisor.	
I know the following emergency numbers: 911; BC Poison Control (1-800-567-8911); MCFD Provincial Centralized Screening (1-800-663-9122); FortisBC Gas Emergency (1-800-663-9911); my Program Coordinator; my Program Manager.	
I will participate in all required emergency drills, training activities, and post-emergency debriefs.	
I understand that this plan is reviewed and updated annually and that I am required to read and sign an updated acknowledgement	



upon each annual update.

Staff Name (print)	
Position	
Program	
Signature	
Date	
Current First Aid / CPR Level C expiry date	
Current OFA Level 2 expiry date (if designated FAA)	
Current WHMIS completion date	
PC Signature (confirms facility orientation completed)	
Date	



**APPENDIX Appendix M — QUARTERLY REVIEW MEMO TEMPLATE**

Completed each January, April, and October per Section 58.1 (the July quarter is covered by the full Annual Review, Appendix K). One page. Filed in the Emergency Preparedness binder. References this plan by Policy Number.

Policy Title	Emergency Preparedness Policy and Plan
Policy Number	ECC-EP-2026-002
Version in Effect	2.0 — July 2026
Facility Name	
Quarter Under Review	Q1 (Jan) / Q2 (Apr) / Q4 (Oct) — circle one
Period Covered	
Date of This Review	
Reviewed By (PC name)	

Quarterly Review Item	Confirmed / Action Needed
Emergency Contact Directory (Appendix B) is current and all numbers verified	
All drill records for the quarter are complete (Appendices G and H)	
Training compliance for the quarter reviewed — no outstanding gaps	
72-hour supply seasonal readiness confirmed (winter additions for Q1/Q4; summer heat readiness reviewed at Q2)	
Youth Emergency Profiles (Appendix F) current for all residents	
Outstanding corrective actions from the prior quarter reviewed — status	



confirmed	
Any legislative, accreditation, or MCFD Standard change identified that affects this plan?	Yes / No — if Yes, describe:
Any material gap identified requiring escalation to PM or an out-of-cycle plan update?	Yes / No — if Yes, describe and confirm PM notified:

PC Signature	
Date	
PM Review Signature and Date (within 5 business days)	

Version History

Version	Date	Summary of Changes
2.0	July 1, 2026	Comprehensive revision of Emergency Preparedness Manual; now titled Emergency Preparedness Policy and Plan; expanded from the prior ECC-EP-2026-001 to include all 7 emergency categories applicable to Canadian residential group homes; full accreditation alignment added (Accreditation Canada Qmentum ROPs; CARF 1.J.1–1.J.6; CCWAF domains); 50 sections in 12 Parts; 13 Appendices; complete 12-month monthly drill and training calendar; accreditation self-assessment tool (Section 59); structured Quarterly Review process added (Section 58.1, Appendix M) alongside the Annual Review; Canadian legislative framework table including federal legislation; all emergency types covered including CBRN, tornado, extreme cold, data breach, media crisis, financial emergency, youth-perpetrated weapons, bomb threat; aligned to BC OHS Reg. Part 4; BC Fire Code Division B Part 2; MCFD SHSS Standard F.4/C.1/G.3; CCALA; CFCSA; Emergency Program Act; Emergency Management Act (Canada); Health Canada communicable disease standards; CDSA
1.0	July 1, 2026	Initial Emergency Preparedness and Business Continuity Manual (ECC-EP-2026-001) — superseded by this version

Signatory	Signature / Date
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Executive Director	Signature: _____ Date: _____
Board Chair	Signature: _____ Date: _____